

ON MY KNEES, I SEE CLEARLY

*A Tale of Cancer Survival and the Power of Prayer
Through Kidney, Spine and Eye Surgeries*

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DEDICATION

This book is dedicated to my wife Julie. She has been by my side for over 24 years, and with her, these trials were bearable.

I want to praise God for his grace. It took five years to be able to explain what three surgeries and their impact meant to me. Thanks also goes to Amy Gingerich for the editing she provided on the book, my daughters and wife for their illustrations, and my good buddies that are comic artists who provided illustrations for the chapters and cover (William Tucci www.williamtucci.com , Andy Lee www.findandy.com and Ed Coutts www.edcouttsart.com).

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Cover by William Tucci

OCTOBER 2000

I fell to my knees in the family room. There was a pain in my stomach that was brief but powerful. This flu is getting to me, I thought. It was Wednesday, so Julie would be home from work in a few hours. I'll get some medicine then. I came home early, feeling like I had the flu and just wanted to rest. I went upstairs and crawled into bed. I curled up in a fetal position and went to sleep. Our home is comfortable and roomy for a family of four. It is a brick two-story with a basement. All of the bedrooms are upstairs. My wife and I use the master bedroom with its own bathroom and the two girls have their own separate bedrooms with a shared bath down the hall. It was quiet and I fell asleep quickly.

Several hours later Julie arrived home from teaching school. She is a math teacher at a local high school.

Julie is a generous and passionate redhead. Julie is loveable and kind and loves cats. She hates being compared to Mrs. Garrett on “The Facts of Life”. Julie and her late sister Millie collected friends the way I collect comic books. I trust Julie with my life and wanted to wait until she got home to take any medicine. She and the girls increase the noise level in the house when they arrive, putting books and backpacks down in chairs, getting drinks of water or a soda, and talking about what happened during the day. Today was no exception, and when Julie came upstairs, she found me in bed.

“Are you sick?” asked Julie.

“I came home early today and just went to bed,” I said.

Julie wanted to know if I had taken any medicine for it and I said no. Dr. Mom was home and she was going to make things better.

“I think I have the flu,” I said.

“Have you eaten something?” she asked.

“No, I just wanted to sleep for a while,” I said.

I ate supper and went to bed, sleeping for about 9 hours. This is typical for me when recovering from an illness. I try to eat a lot of food, stay warm and sleep as much as possible. I absolutely hate going to the doctor.

The following Tuesday Julie was in Florida visiting her brother. Julie insisted I make an appointment with Ms. Powers, the nurse practitioner, or Dr. Sarah O'Reilly, our family doctor. They could see me next week on Friday. Thursday was when Julie got home from Florida. For a week and one-half, I went to work and only stayed home one-half of one day. I sat in

meetings and at one point was physically there but not mentally. I later found out that people said I looked yellow or green or gray in the face.

Dr. O'Reilly has an office off of the back of a local hospital. She is always smiling. She always wears a white lab coat and has a stethoscope around her neck. She has some freckles and just seems like a normal person, complete with normal problems, just like me. Julie and I went to her office on that Friday back in October.

After waiting about 20 minutes in the waiting room, I was called in. It does not bother me to have Julie in the examining room, since we had been married 18 years at that point. Dr. O'Reilly and Ms. Powers had the rooms set up with a couple of plain chairs, a chair with rollers on it and an examination bed. There are always tongue depressors and latex gloves on the vanity and a medical waste bin for her garbage. Charts on the

will help to pass the time waiting for the doctor once you get into the examination room.

Ms. Powers is the nurse practitioner. She saw me first. She is a smart and caring lady. She knows her stuff and wants to give excellent care to all the patients. I have been impressed with her.

Ms. Powers said, “Mr. Snider, how are you today?”

“Nurse, I have been feeling bad for about a week or so and this flu is not going away, so I thought I had better come in,” I said.

Ms. Powers poked around, looked at my throat, took my blood pressure, looked in my ears, banged my knee and conducted routine kinds of tests. She evidently did not find anything doing that, so she wanted me to get up on the table.

“Mr. Snider, does it hurt anywhere?” she said.

“No, not right now, but two Wednesday’s ago, I had a big pain in my stomach that took me to my knees. It was only a second, but it hurt quite a bit at the time.”

Ms. Powers pushed into my stomach area with her hands, feeling around for anything that might be wrong. I thought she was checking for an intestinal blockage or how full my stomach was. She never gave me a clue that anything was wrong but said she wanted Dr. O’Reilly to take a quick look. Ms. Powers left the room and I did not think anything of it. She then asked Dr. O’Reilly to come in. Dr. O’Reilly repeated a lot of what Ms. Powers had done and had me lie back and felt my stomach area. Dr. O’Reilly wrote down some information on the chart and then went out of the room. I thought she would tell Ms. Powers to give me a shot and send me home. Instead, Dr. O’Reilly came back in and said she wanted to order a blood test and CAT-scan plus x-rays for my abdomen area.

“I need to see more and these tests will tell me what to do next. It is not the flu.”

Looking for a quick fix, I just wanted some antibiotics and maybe a shot and to go home.

“When can we schedule these tests?” I asked.

She said she had ordered them to be scheduled right now in the adjacent hospital. I figured doctors had some clout at hospitals and this was evidence of it. I had no idea at the time that it was serious business and an emergency to get the tests done. If I were to schedule tests on my own, it would have been many days before getting the tests done. Julie and I were instructed to come back to her office after the tests were run. The blood test was done by the phlebotomist (someone who drains your blood and seems like they enjoy doing it). I have to look away when people use needles on me. I can handle it without fainting or

anything, but prefer not to look. If it is anyone else's blood, I can help out and think nothing of it. After the blood was drawn, we went over to the hospital.

At first I had the X-ray taken. I was getting weaker. Once that was over, I had to go over for a CAT-scan. I had never had a CAT-scan before. It is also called a CT scan based on some of the signs I saw. The process was to drink these two bottles of orange flavored barium drink and wait about 30 to 60 minutes and get the scan done. This type of scan is very similar to having a MRI done, with your body in a tube.

I am so glad that Julie was with me. By this time, the kids needed to have us home, so Julie called and got them to go to a neighbor and wait for us to get home. It was time to drink my first barium solution. When you first taste it through the enormous opening in the thick straw, you have to fight a gag reflex. This is not fun. I can't believe I have to drink two of these

bottles. I hold my nose. I drink a lot at one time. I made the same noise that Bart Simpson makes when rubbing his aunt's feet on television. I looked for a plant nearby I could pour the drink into, but I knew it was for my own good, so I didn't throw it in the plant container. If we can send a man to the moon, can't we do something about this drink and how awful it is?

In waiting for the barium solution to run through my stomach and kidneys, etc., I was walking around and started to lose all of my energy to stand up without a prop. I used the handrail in the hall to keep standing. The time finally came to have the scan done and an assistant helped me into a dressing room. I was supposed to change into a hospital gown and then go into the scanning room. I was led into a dressing room area and the attendant hung up a gown for me to change into on a hook. I wanted to be a good patient. I really did. My body was not cooperating and my

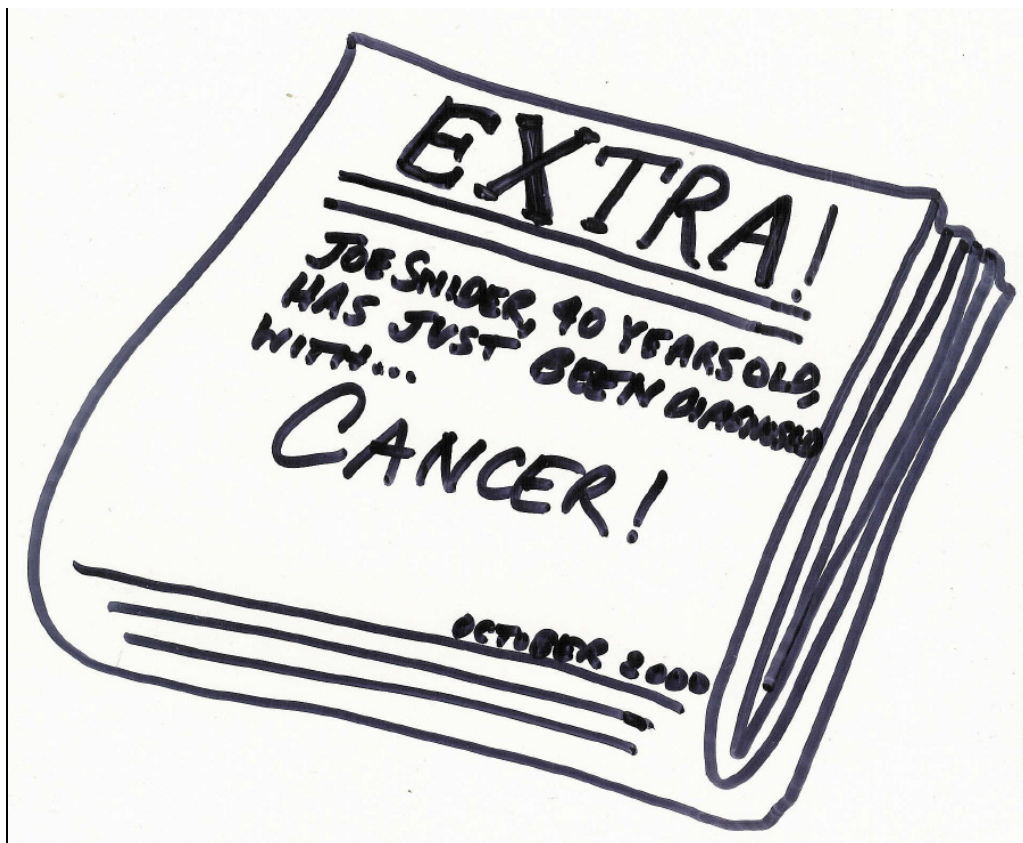
mind was very unclear at this point. I just sat there. About five to ten minutes later, the same attendant came back and asked me if I was finished. He was surprised when I had not moved at all. I apologized for not being able to change clothes and I felt like a failure. The man was very nice to me and said just to come on in and they would take the scan anyway. He told me to lie down on the table. Since I was also having a “contrast” done, an IV had to be put in. Another first!

Somehow I had escaped broken bones as a kid or the need to have any other major medical procedure until I was forty years old. The IV needle was put into my arm. This is where the “contrast” solution would enter my body. For the first part, I had to lay back, get automatically conveyed back into a tube (they asked if I was afraid of cramped spaces), and watch out for the IV connected to me. I had to do breathing exercises and hold my breath while the scan was taking place. At this

point I was about to pass out, but held on until the end. A regular scan is 20 minutes. It is 40 minutes with a “contrast” being ordered. The scan can show fluid and how it moves through your body. The nasty barium drink flows through first and then the IV solution. When it came time to receive the IV solution, the nurse said I would feel warm. She wasn’t kidding. I felt it go in and a few seconds later felt a warm feeling in my bladder. I felt like I would urinate all over myself right then and there. The process was over and I started to gain back some strength and presence of mind by being able to lie there, motionless. I got up off of the table on my own steam. I felt like I had just finished a 10 mile run and really accomplished something getting off of the table by myself. I went back to see Julie in the waiting room.

Julie and I went back to the doctor’s office and waited in the waiting room for about 30 minutes. By

now, the doctor was starting to receive results from the blood test and CAT-scan. We were called into an examination room and waited for the doctor to come in with some news. At this point I was wondering when I would receive my antibiotics and go home to feel better. Dr. O'Reilly stepped in with a folder in her hand, looking down at it. Julie and I both asked about the results, almost in unison.



"The News"

THE NEWS

Dr. O'Reilly said, "Mr. Snider, we are going to need to admit you into the hospital. You have a tumor around your left kidney and it is more than likely cancerous and is going to need to come out."

I am glad I was sitting down already. I HAVE CANCER. My mind was racing. So many images were coming into my head so fast that it was hard to focus. I thought about my daughters and wanting to hug them and hearing that they love me. I often imagine rocking their future babies on my lap, cradling them in my arms and having them then fall asleep on my belly on the couch, so peacefully. I have always envisioned myself as a grandfather, hoping to be like my grandfather George Lake. Grandpa was a kind man. He held out his hand in the back yard and squirrels would come up to eat out of it. This is what I think St. Francis must have been like. Grandpa never had a mean word to say

to anyone. He loved his garden and fruit trees and his dog. Most of all, I could tell he loved me. That is what I want my future grandchildren to feel about me. An image darted through my mind of my mom wanting a hug from her chair. She was the only parent I had left at this time. My father, grandmother and grandfather died many years prior. I saw my home. I saw the church. I saw Al and Etta, who are close friends in the church. Al and Etta are the salt of the earth and members of my church. They were in their early eighties at that stage of my life. I saw my close neighbors and thought of the 4th of July and New Year's block parties. I thought about my co-workers, Steve, Susie I., Susan, and the Susie R., Deb, Bonnie, Eric, Tom, Neal. I had no past and no future. I only had this moment.

Even though it had only been a second, all of those images went through my head.

I reached out for Julie's hand and asked the doctor, "Is it malignant? Is it localized to my kidney? Is the other kidney OK?"

Dr. O'Reilly said, "We won't know more until they go in and find out more. I will get the operation scheduled and you will need to go down to admissions."

Dr. O'Reilly left the room, saying she would be right back.

Julie said, "You are being very calm about this. I would be going nuts."

"I'm forty years old and going to lose a kidney." I said.

I hugged her and started to cry. I quickly regained composure, not wanting the doctor to see me crying. This is the first time as a man that I ever cried. I am glad it did not last long. I sat back into my chair,

waiting for the doctor to reappear. I asked the lord to take this problem and I would try to follow his lead, being a good patient for the doctors and a good person for him. The burden was suddenly lifted from me at that split second and I was never scared about it again. From the time “THE NEWS” was given to me until this moment was around 15 minutes. The answer to the prayer for peace was immediate and long-lasting.

I was put into a wheelchair and wheeled down to admissions. This was part of the policy and procedures when someone was being admitted to have surgery. We got a room number to go to that was being prepared for me. Julie wheeled me up to the room and when we got there, the bulletin board was empty except for a business card. It was from my preacher, Ken Hughes. He had been there and wanted to let me know it. We were in the right place. He and his wife are good friends of ours. Their kids also get along with

our kids very well. Someone cared! I got up in the bed and started to rest. By that time, whatever was wrong with me had taken hold, and I was feeling pretty bad. It must have been about an hour before the doctor showed up. During that time, an IV was put in. They wanted to get the biggest gauge opening possible for the IV. I do not know why, but they made a big deal out of it. The one hand did not cooperate, so they tried the other hand. It worked fine. Then the doctor walked in.

“Hello Mr. uh...Snider, how are you feeling?” said the doctor.

I told him, “a little groggy, but I want to know what is going to happen.”

Dr. Bricks started drawing on a piece of paper in his tablet a picture of what looked like a balloon. That represented my kidney. Dr. Bricks was an older man

with silver hair, kind of large and business-like. He wore a lab coat and carried a tablet on a clipboard. You could tell he did not want to rush, but also had a lot of work to do. It was also the weekend, Friday night, and I am sure he had plans. He then drew a dark area towards the middle of the balloon.

Dr. Bricks said, “You have a naval orange size mass blocking the flow of urine from the left kidney. The kidney ruptured, leaking urine into the body cavity.”

He drew little waving streaks coming out of the balloon kidney. I knew this was a toxic substance running around in my belly.

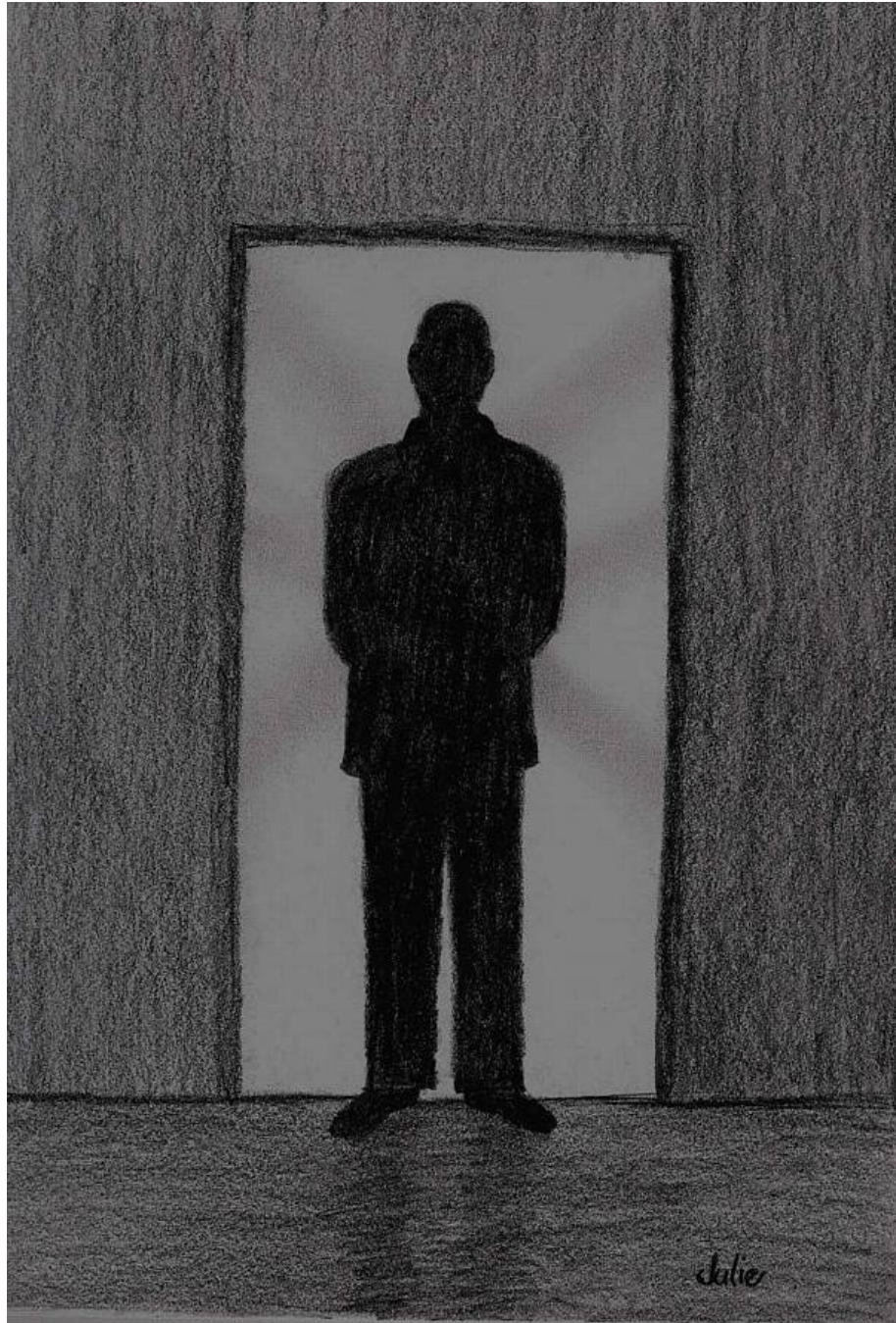
Dr. Bricks said, “This caused peritonitis. Infections in your abdomen are the result. We are going to go in tomorrow morning and take a look. I hope we do not have to remove the kidney, but we might. You need to rest right now. Do you have any questions?”

I asked, “Why did it hurt so bad that first Wednesday?”

He answered, “Well, the pain was probably the blood flow in and out of the tumor. We can’t tell from the scan if it is merged with the kidney or attached to the kidney.”

I was sure I should ask a dozen more questions, but I was so tired by then and needed to sleep. Dr. Bricks left Julie and I alone to talk about things.

Julie called for our pastor and the message said he was out on vacation. Wait a minute. How could that be, when we saw his card on our door? We found out later that our pastor had not visited that room but somehow it was the only card on the bulletin board in the room. That is the Lord’s doing. We were able to get in contact with Sonny Wilson, and he contacted Ken Hughes.



“Angel in the Doorway”

It was not too long, and Jim Shelburn appeared in the doorway. He was our youth pastor back then. Julie saw him in the doorway with the light behind him making him appear like an angel from heaven.

“Brother Jim!” she cried.

She went to give him a hug and broke down in front of him, which was quite a sight with her head coming up to his stomach. Jim was and is a big man. He just comforted her and let her go ahead. He came to pray with us. Jim is an extraordinary preacher and now has his own church, which he leads. All the kids respected him and had fun. Jim was an ex-marine and used loving discipline to motivate kids. Jim came over to the bed, we held hands and he prayed. Our hands were about half the size of Jim’s hands. I remember thinking about meeting and shaking hands early in my career

with Ken Olsen. Ken Olsen was the CEO of Digital Equipment Corporation, many years prior. The man was built like a kodiak bear and had the paws to prove it. My hand fit neatly inside his too. It is funny how your mind drifts when you are trying to focus.

There are few people in this world that have a gift when it comes to praying out loud. Jim is one of them. Another is Wood Bailey in my men's group at the church. Wood was in his 80's at the time and has since passed away. You could hear him say Amen during sermons. He did not care who heard him. You can tell when these guys pray that it is spirit-filled and inspired. I felt honored by the gift.

The preacher, Ken Hughes, and his family also came by to see us. He is a good friend and we have a great time telling jokes and causing trouble annoying our wives in the bargain. Ken forfeited his vacation to be with me. He prayed with us also.

My family doctor and nurse practitioner also came by to see me. They talked mostly to Julie and were surprised that surgery had not taken place. I was happy to see them but also pretty out of it due to the drugs in the IV, so I basically just said hello. They mentioned that I had a high tolerance for pain and should have been flat on my back a week ago. They told me that if I ever had pains to go to the doctor immediately, since I did not react like normal people to symptoms. They were very surprised that I was walking around, driving and going to work that week.

Julie went home to be with the girls. She called her sister Alyis and told her about everything that had gone on over the last day. She bawled on the phone like a baby. Julie just lost her older sister Millie to cancer one and one half months earlier. Millie was 48. I guess Julie loves me and wants me around for a while. She also called my boss who was very worried, since he only

knew that I left to go to a doctor's appointment on Friday. Julie and I also had to call on our good neighbors, the Gabhart's and Abondolo's for feeding our girls, taking them to school and being there for us during my hospital stay.

That evening, my temperature was rising. I have been known over the years to have very high fevers when sick. This was no exception. I had several "flash fevers" I call them and I would sweat over 90 percent of my body. Of course when this happens, the sheets get soaked and need to be changed. The nurses were putting in quite a bit of time on me that evening. Changing sheets, helping me to the bathroom, dispensing the medicine through the IV, monitoring my urine output, taking blood for blood tests every 4 hours and general vitals measurements. The blood and urine checks were to check the creatinine levels which show how well the kidneys are working. Also my white

cell count was extremely high due to fighting the infection. Julie and I later learned that my blood levels were indicating that I had about 24 to 48 hours to live if no medical treatment was provided. I felt so sorry for all the trouble I was causing the nurses. They were all so nice to me. It was hard to sleep that night due to all the information I was handed, the hourly visits by nurses, and waking up in a total sweat.

The next morning it was time to go to pre-op. There they begin all of the procedures to get you ready for an operation or outpatient medical procedure. I was rolled down in the same bed I slept in. That was kind of fun actually.

My first hospital procedure! I met the anesthesiologist. The nurse shaved hair on my leg to put leads on them for an electrocardiogram. I was given some medicine through an IV (now old hat) to make me loopy and relaxed. I thought about getting a

marker and putting an X where not to cut on my right side because I had heard of doctors taking out the wrong organs before. No markers were available. I remember being rolled into the surgery room and the anesthesiologist was in back of me, the urologist was in front of me, nurses were to the side and in front of me and people were milling around. I thought to myself, I should have sold tickets, especially since they were flashing me to the world right now! They told me to count backwards from 100. Since they embarrassed me by flashing me to the world, I wanted to be stubborn and stay awake as long as possible during the countdown. I made it to 94 and then was out. Another first! In the time it took to count down, the urologist was already working on me, putting something up my penis. That was a first I could have lived without!

Later that morning I woke up in the hospital room.

I had no sense of the time that had elapsed, but thought

it might be the same day. Julie was there and I remember a set of flowers on the table across from me. I started to take a quick inventory of body parts. I checked for my hands, legs, ears, nose, and private parts. It's all there. Good. It wasn't too long and Dr. Bricks came in.

I said, "Hello doctor. Come in."

"How was your night?" he asked.

"Well, I didn't get much sleep and had some fevers last night. I did finally get some sleep though. I feel so tired."

The doctor said, "That is normal for what you are going through."

"So what happened?" I said.

He replied, "We put in a stent between your kidney and bladder. You are going to feel like going to the bathroom a lot while the stent pushes on your bladder."

You are on muscle-relaxers and pain medication, along with very strong doses of antibiotics to take care of the infection. The stent is going to lower the fluid in your kidney and reduce its size.”

“How long will I be in the hospital?” I asked.

“Oh, we will want you here several days. We want you to get to the point where you can eat solid food again.”

This sounded bad for a man over 210 pounds. This kind of diet was not in my plan.

“Why didn’t the kidney get removed?” I asked.

“We do not want to do that. Instead, we would like to see you get through this infection, and let’s see if strong antibiotics and steroids will help reduce the tumor size. Even if it has to come out later, it should be smaller and an easier operation to perform. Do you have any more questions, Mr. Snider?”

Not really being prepared I said, “How about you Julie?” She didn’t either.

“Oh, Mr. Snider, I do have one more thing. My office does not take your insurance so I will not be able to be your surgeon going forward. There are plenty of urologists in the city for you to choose from, so you will need to contact your insurance and set up a follow-up exam with one of them in your plan.”

Julie was the only one capable of acting on this information in the room, so she was listening intently.

“How will you be paid then?” I asked Dr. Bricks.

“Most insurance providers make exceptions in extreme circumstances. Since you were not able to choose your surgeon at the time, this should qualify. You will need to contact your insurance provider and let them know.”

All I could think was that if I were alone in the hospital, and under heavy drugs, this would be a real downer, versus just a small downer. The surgeon left the room, saying his office would either have him or another person check on me tomorrow. I was now in the hospital with no assigned doctor to take care of me. I have a tumor that Dr. Bricks did not want to operate on. I think he believed that his fee would not be paid. I wondered if the decision not to operate was in my best interest or his. This is where a medical advocate system would be an advantage. If people do not understand the jargon, are too sick to deal with the issue, or just do not believe the diagnosis or want someone to explain in more detail than a 10 minute office or hospital visit can accommodate, then someone who can play that role would help immensely.

My stomach hurt. The kidney is actually protected by the bottom area of the rib cage. My ribs were sore

also. Moving around felt like crap. Having your life worth measured in terms of milliliters of urine was demeaning and reduced me to a statistic. I was pleasant to all that came into the room, because I had promised the Lord I would as my part of the deal. My job was to be a good patient. I could feel something funny at the top of my bladder – that was where the stent, also called a shunt, would press on my bladder, making it feel as if I had to use the restroom.

That day and night I did not get much sleep due to the pain. I had different strengths of medicine given to me and it did not seem to make a dent – similar to having a nagging toothache before getting relief at a dentist. The difference was this was my abdomen, but it still kept me awake. The doctor's orders were to get plenty of rest to fight the infection.

The nurses would ask, “From one to ten how is the pain?”

I answered, "It is around eight or nine. It mostly keeps me awake."

Then the nurse would go back to her station and leave me alone until more vitals were needed, or I needed medicine. I was alone in the dark room still not able to sleep. It was around 2:00 the following morning. The doctor had to be called for any other medication to be given. I said I could not go to sleep due to the pain. The nurses were intent upon following orders and called the doctor to get his recommendation. The nurse came in with a shot to put into the IV. She could insert the needle into the tubes directly versus inserting it into me. That made me happy. The nurse said it was two milligrams (I think) of morphine direct into my bloodstream. After putting it in, the nurse said good night and left the room. I relaxed. It was fast-acting. I felt my head get numb from the temples back to the back of my skull. I think it was putting my brain

to sleep. I have since felt the same sensation from BBQ sauce with a lot of vinegar in it. After a couple of minutes with my head numb, I got on the buzzer for the nurse.

She came on the intercom, “How can I help you Mr. Snider?”

“I just wanted to thank you. You are all angels!”

The nurse laughingly said, “OK Mr. Snider, you have a good night’s sleep now.” The nurse had to have been holding back a laugh.

“OK, bye,” I said.

I slept until morning. I am sure they must have thought I was crazy. Later that night, they took my blood, but I did not remember it.

I was about to bust when I woke up. I could get up by using my arms to pull myself up, and immediately went into the bathroom. I almost did not use the

measuring cup and caught myself. I wanted credit. I was being measured. My life depended on it. I needed to hear “Good job!” from the nurse. The stent was certainly working. I felt like I had to go about every 30 minutes. This was of course all regulated by the medicine I was being given. It was difficult to hold the cup and stand up at the same time. This was exhausting! It is amazing to me what a human life can be reduced to, that getting up to a certain measurement level in a urine cup was the most exciting part of the day.

Julie came in not too long after my bathroom break, and asked how I felt. I told the story about the morphine and felt stupid about calling the nurses angels.

“They must think I am stupid.” I said.

“They know it was the morphine. It is funny, though,” said Julie.

As we started to find out more about how sick I actually was due to the peritonitis, we felt lucky that I got into the doctor’s office, that they recognized the issue, ran the proper tests, and got me in the hospital so quickly. Ms. Powers and Dr. O’Reilly saved my life. I hadn’t ever been able to say those words before. If you think about it, how many times do you think that a person is a life-saver, but it is meant figuratively? This was the real deal.

I suddenly had a lot of time on my hands. I wanted to call people, worry about projects and make progress. The only thing I could really do was lay there. I know a woman at work who had to be bed-ridden during the last six months of her pregnancy. I do not know how she stood it.

I asked about a breathing device that helped to stop pneumonia and lungs having a fluid build-up. My sister brought one of these home when I was a youngster. We all played with it, blowing our air into a tube and watching water flow into another area of the plastic. The nurses found one and put it on the meal tray. It was a little different in that it had a red plastic indicator of how much air you could blow out. The more you could blow, the better off you were. At least it was something to do. I read magazines over and over again, went to the restroom, watched a lot of TV, and slept.

My meals kept getting more and more substantial. Within the third and fourth days, I was able to eat normal food again. What a relief! This was some visible evidence of getting better and being able to leave the hospital. I had difficulty with my bowels working, so that was the last step in the process to

going home. More evidence to check off was being able to walk around the floor, first with the IV attached to me and then without it. It took four and one half days to get over the peritonitis well enough to go home. I had lost 30 pounds in five days. I do not recommend this diet to anyone.

I received a visit from my team while I was in the hospital. That cheered me up. I could tell some were very uncomfortable. They brought me some magazines to read and a card. It was a social call. Some felt obligated, but others did truly care.



Compilation of Personal Church Photos and
“Laying Hands” © Jean Kimball at www.BigStockPhoto.com

NOVEMBER 2000

I was in the hospital from October 20, 2000 to October 24, 2000. In that time, my weight dropped 15 percent, or 30 pounds, and I was feeling weak still. It would soon be time to go back to work. I went back about a week after getting out of the hospital, October 30th.

The major events of November were getting a doctor, going to church, going to work, having an MRI done versus a CAT-scan, and seeing the new doctor about my condition.

I got home in time for Halloween; kids came to the house for trick or treat. I was basically out for one week. As always, I went back to work before I probably should have. My next appointment was before

Thanksgiving, and I had another scan done to tell if the tumor and kidney size was smaller.

I decided to do my own research while on my lunch hour or at home. This is a good step for anyone who has questions and no doctor in the family. I looked up peritonitis on the internet. I found the following information on the webmd site:

Peritonitis is an infection of the lining of the abdominal wall (peritoneum) caused by bacteria or irritating substances. Severe peritonitis can be life-threatening. Signs of an infection in the abdominal cavity (peritonitis) include:

- Swelling of the abdomen.
- Severe pain and tenderness in the abdomen that becomes worse when the person moves, coughs, or presses on the abdomen. The pain sometimes extends (radiates) to the shoulder.
- Nausea and vomiting.
- A rapid pulse.
- Chills and fever.
- Rapid breathing.

A person with these symptoms should seek immediate medical attention. The person

could become very ill and possibly die within hours if he or she does not receive treatment.

This explained the situation more thoroughly than the doctors did. I had everything but the vomiting and rapid breathing. I added loss of concentration and loss of walking to my list.

I was also interested in what a stent was and renal failures, nephrectomy, kidney cancer, tumors and, most importantly, finding a new urologist to be my doctor.

A stent is a small tube with a pig tail on the end to make it stay put. This was placed between my kidney and bladder and caused me to need to urinate frequently. I was taking bladder spasm medicine to help with that problem. It felt like my wife describes her eighth month of pregnancy with our two daughters, when she had the same issue. Renal failures are

basically kidney failures and a nephrectomy is kidney removal surgery.

There were at least two ways to remove a kidney or kidney tumor. One technique is to use small holes in the abdomen and remove organs or tissue from those small holes. The other is much more invasive and requires a 12-inch incision below the ribs. The surgeon would remove the organs and tissue much like a C-section when delivering a baby.

My wife and I found Dr. Short in the phone book. While not a scientific or possibly even a good way to locate a doctor, we had to start somewhere. Now there are online services like www.jcms.org that help in locating doctors in Louisville. That would have helped a lot. Dr. Short's office was close to Baptist East Hospital which is where I wanted to go for the surgery when it was time to have it. Dr. Short was taking patients and took our insurance, which was a must.

There are different hospitals in Louisville that specialize in certain things. This is probably the same anywhere in the country with multiple hospitals around. For back pain you go one place. For heart trouble you go another. For cancer you can go to several places, and it depends on where it is in your body. For having babies, foot problems, pacemakers, ear nose and throat, and even hand surgery you go to different places depending on what is wrong. Baptist East is known for kidney surgery.

We set up the appointment with Dr. Short's office and he took us in. I felt like one battle was over. At least I had a doctor instead of being in limbo.

Earlier, I mentioned that I gave my problem over to God. God gave me the peace to not be scared. Five years later, when I write this, I can honestly say I was scared only for that initial 15 minutes after being told I had cancer. I knew from that moment this was not

going to be a life and death situation. Being unafraid is important because you need to be clear-headed about treatments and medical care. God gave me a brain to manage my own health care, which is what you have to do these days. God gave me the physical means to fight the peritonitis in my body and good health care provided mostly by the nursing community. God gave me the strength of will to know if a task was painful, to get through it. God gave me a sense of humor and patience, which were taxed during the four months I dealt with the kidney problem. God gave me a sense of purpose, a family who depended on me, and my Mt. Tabor United Methodist church family, to fall back on. God provided me with a loving wife who took on much of the burden without complaining. God was also going to provide me with a greater sense of humility during this time and an ability to accept help. I did not realize it at that time. That realization came much

later. God provided souls that prayed for me, and I do not even know who some of them were to this day. I praise him for his wisdom and healing. I say wisdom, because if I knew the pain involved, I would have worried about it. As it was, I lived through it. If I knew the costs involved, I would have worried about that a lot more. God provided the information in his time, not mine. That was a great lesson.

During November, my weight picked back up a little, about 10 pounds. I actually felt pretty good and went to work and to look at me, nobody could tell that I was going to need major surgery. Many people knew about it though. There was my family, my wife's family, our church, people at work, and even my brother-in-law's church in Florida. Our pastor was and still is a good friend, even though he has since moved on to bigger and brighter career goals. I went to church one Sunday and people were happy to see me come

back and sit in the pew. I wanted to take advantage of any opportunity to get well or have God's healing in my life, so the pastor had a special healing service where I went up to the altar, he laid hands on me, placed a cross of oil from the Holy Land on my forehead, and invited others in the church to show their support at the altar.

A flood of folks came up from their seats. Two-thirds of the people came up to put their hands either on me or on someone touching me. There must have been 40 people at the altar. This was very humbling, and I felt electric. I felt hot and cold at the same time, and I was moved emotionally. Our church is Methodist and is pretty conservative, so this is not a sight you see often. Our church is not evangelical. Our church family does care and is devoted to prayer and healing and the supporting of people in need.

I was in need. That was hard for me. I had to lay aside pride and learn how to accept other people's

kindness and charity. My family was always poor growing up. Since I was in the seventh grade, any extra food, clothing, comics or gas money had to be earned through lawn mowing around the neighborhood. Otherwise, those extras did not exist. Our neighborhood was the housing projects of Danville, Illinois. In the housing projects, there was a maintenance shed that my brother and I went to. It was OK to take a mower and a shovel out for the day to try to earn money. I mowed and my brother trimmed the sidewalk with the shovel and then on the next yard we switched off. We were the best in the neighborhood, and fast too. The mower was a metal blade human-powered mower. Home Depot now sells light-weight metal versions for small yards, but back then we could not afford the gas for a gas-powered mower and self propelled mowers were not affordable. The older ladies would pay us one dollar to mow the yard and trim the

sidewalk. With the shovel we would do what blade trimmers do today, but we did it by hand, several inches at a time, on both sides of the walk.

In school, I would always be the one to help others in their homework. It came easily to me. In 4th grade I was doing 6th grade math independent study and showing others how to do the work to help out the teacher, Mrs. Rogers. In 6th grade I asked the history teacher to tell me of an interesting history text that would be high school or college level reading. He gave me a 1,000 page book and I read it with a dictionary beside me to look up the words I did not know. I gave it back to him in one month, having read it at home. Taking education into my own hands was normal for me due to the example from my older brother Ivan. He was my role model. Ivan paid his way through college by doing drafting work and was valedictorian of his high school of 2,000 students.

I paid for my college education by working summers finding gas leaks for power companies. The old timers used matches to measure the severity of the leaks. I used a metal rod to poke holes in the ground or pavement, measured the leak with a sniffing device, and then recorded a map on a survey sheet of how to find the leak and get it repaired later. I traveled town to town, since the jobs only lasted a couple of weeks for any given city or town. The jobs were great, but I was alone and had to fend for myself. I had to find cheap housing in each city. I had to find a bank to cash checks, and find where the office was on my own. I found food and clothing and entertainment. I played on tennis courts in the evenings, or I would ride my ten-speed. I even went to various churches on my own, testing them out, finding out which ones made sense to me. I was bold enough to get baptized in a lake in Illinois when I was 19 years old. I tell you this so that

you can know how hard it was for me to accept help from anyone.

When I went to college on my own funding and fierce independent nature, I did not need others to survive. I did pretty well on my own versus living in projects or public housing like when I was a kid. While going to school, I held down jobs, financed a car and insurance, earned good grades, and found time to socialize with friends in college. I was on top of the world, eating Panteras Pizza and playing Pac-Man at the local pizza restaurant. The only person I could not be without was Julie, my then-girlfriend and now wife. That was true back in those college days and has been every day since then.

At 40 years plus three months, I needed people. I needed God and my church family. I needed to praise God. I needed to forget about being embarrassed being in front of the church congregation with a hand

out looking for healing. The words “hand out” still make me feel like I was looking for money or something, and it was to be avoided at all costs. What I learned through this experience was to accept other people’s kindness with humility and their service and prayer with thankfulness. Being on the receiving end felt funny, but that was part of the lesson. It was God’s lesson. It is truly character-building to find out you do not know everything, that you need help from time to time and it is OK to receive it. Others in the church said it helped them too. When you give and do not expect anything in return, that is when the Lord steps in and blesses the gift. In this case it was not a material gift, but a spiritual one.

Days passed, and I went to work on a regular schedule. I came home, ate supper, and went to the bathroom five or six times every evening. Things were progressing. At work, I could do everything I needed

to. I managed a group of programmers, so there wasn't heavy lifting or harsh activity involved. I attended meetings, and could concentrate on each job to do because being scared was a non-issue. Not all of my worries went away, since I did have a family to take care of.

This brings up a point about life insurance. My family would have been financially better off, especially in the short run, if I had died from this cancer immediately or at the hospital. The home and cars would be paid off. The college would be paid for. My wife would have the choice of working or not working. This notion, fleeting as it was, did occur to me. I believe the notion was put in my mind in a moment of weakness. Then my mind came up with alternatives to that theory. In the long run, the income from my job would more than outweigh the insurance now. If my wife did not re-marry, any grandchildren would not

have a grandfather to help raise them. My wife had been with me for over 20 years, between dating and marriage, and she would have to train someone new. That would not be fun. It takes a good five years or more for a man to be properly trained to be acceptable to a woman living in the same house. Some guys never learn. What it boils down to is that my wife loves me and wanted me to be here with her. It is as simple as that. No more mental discussion. Case closed.

At work, I appreciated people a lot more. It was a strong lesson from this experience that it only takes a couple of hours and you might not see them any more. I like to mentor and teach people. I want them to grow in their experience and in getting through issues and problems. I want to make constructive progress, not gloss over issues or shove them under the carpet. I am fortunate in that the corporate culture is one that is similar to my views in that people come first. People

capability is important and must be given its due in terms of training and development.

Priorities come into focus more clearly when you are in limbo. Time stands still. Normally your view of the world is clouded or muddy. Not when you are in limbo. This must be a sort of purgatory. You have a lot of time to think and reflect. Front and center are the members of your family and other people with whom you have constant contact with. My sister-in-law Millie was the oldest of my wife's siblings. She had colon cancer and died from it a few months before my illness. Before she died though, she had a resurgence of life, went home, and had just enough time to see her only daughter get married. That was no coincidence. God needed Millie. At the funeral service, it was said that she collected friends like some people collected coins or stamps or toys. Everyone she touched loved her humor and good graces. Over 250 people attended her funeral. I had

only seen that much interest at my grandfather's funeral and he was over 70 years old and knew more people than Millie did. I remembered my wife getting married to me, then losing her father and then her grandfather all in the summer of 1982. Those feelings were coming back to her and she did not want to relive that series of events again with different faces.

The priority of the Lord in my life changed during this period as well. He was working on me to become a better Christian. I went to church more. I prayed more. I praised God. I read the bible more. I said the Lord's name more. That sounds simple, but if you count the times you say Jesus in one day, it is not very many unless you are a minister or priest. Focus on what was meaningful in my life was easier in limbo.

Bible verses started having more meaning to me at this time. I like the book of John and always have. I

pay close attention to the red text, considering it more closely since that means Jesus said it.

John 6:2 says, “And a multitude followed him, because they saw the signs which he did on those who were diseased. (Revised Standard Version).” Also, the book of Luke has some verses that relate to disease.

Luke 7:21 says, “In that hour he cured many of diseases and plagues and evil spirits, and on many that were blind he bestowed sight. (RSV)”

Luke 9:1 says, “And he called the twelve together and gave them power and authority over all demons and to cure diseases. (RSV)”

“Could Jesus cure me?” I thought. All the Sunday school I attended said yes. All the preaching I heard said yes. My prayer said yes. What did a cure mean? Was my calm attitude the cure? Was the fact that my wife wanted to help me through this the miracle? How

much should I rely on doctors and how much on the Lord?

My prayer-answer was that I should take every opportunity afforded to me to get better. To me that meant getting good medical care, eating right, sleeping, praying, reading the bible, going to church, being willing to have people lay hands on me at church, trying to make Julie not worry so much, being around my family and friends and working at my job. I have always had a vision of being a grandfather, having a baby sleeping on my stomach as I lay on the couch, all nice and warm. That is such a clear vision to me that it must be true that it will happen. This has been in my head for over 20 years. It is so clear that I call it a memory, not a vision.

Reality hits like a brick. The bills for the first hospital stay started coming in. The hospital had a bill. The anesthesiologist had a bill. The surgeon had a bill.

I think the maintenance engineer at night sent us a bill for goodness sake. Julie did not mention a lot of that to me at the time. She takes care of the bills and always has in our marriage. Maybe she didn't want me worrying about it.

It came time to have an MRI and a doctor's appointment with my urologist, Dr. Short. The MRI shows different pictures, although it is similar to a CAT-scan. Many doctors prefer one over the other and sometimes require both of them. In either case, they still put your body in a tube and use a conveyor to move you into it. A lot of people get claustrophobic inside these machines. I never had any trouble as long as I could see out of the back of the machine and see light.

I went to the test and was much more curious and certainly more with-it mentally. When I had the first CAT-scan, I was about ready to fall down and could not focus my mind at all on the simplest of tasks like

getting into a hospital gown. The orange barium solution was still awful. That did not change. It tastes like orange plus chalk plus metal. It is not fun. It helps in the diagnosis by showing up as lighter than your organs. What did change was the fact that I could see the results after the test through the window. I went through the test with flying colors. I was a good patient when the contrast solution was being put into my vein, during the breathing that needed to happen on cue while I was in the tube, and I followed all the instructions to the letter. I was doing my job. After the test, I went over to the window and could see the pictures on the 21-inch computer monitor. I asked where my kidney was in the picture, and they pointed it out. It looked two and one-half times the size of the other normal kidney.

“Where is the tumor?” I asked.

The technician pointed to it. It was in the middle of the kidney like a fist choking a water balloon.

“Is it connected to the kidney or resting on it?” I asked.

“You will need to speak to your doctor about that,” said the technician.

“OK. Thanks for showing me.”

“Your doctor will get these after the radiologist goes over them and passes them along.”

I wish that Julie could have seen this. She is really good at spotting things. We will be walking along in our lawn, and if any clover is nearby she will be able to find several four-leaf clovers quickly and I will not find a single one.

I trust Julie’s judgment and want her to participate in each part of the decision-making process. I do not trust people easily. That might come from living in a

pretty dangerous part of Danville, Illinois growing up. In the housing projects you get exposure to drugs, can get beaten up, and have things stolen from you or worse. Becoming a criminal was very common in those projects. You learned not to trust too many people.

We moved from Danville when I was a teenager. I felt more freedom and could trust people more in the new town we moved to. Georgetown was a much better place than Danville to grow up. It was smaller and people were much nicer. I could walk to school or the grocery store. Our neighbors were not dangerous to be around. I found friends with the geeky crowd and remained friends with them all through high school. My grandmother and grandfather lived in a small house in Georgetown, so I could be around them. I liked that.

My grandma and grandpa were named Daisy and George Lake. They lived together in a tiny house worth about \$6,000 back then. My grandma made the best

homemade sugar cookies when we would come over and stay the afternoon. They were happy and he and I grew a vegetable garden. They always seemed to have cats around too. We could never have a cat where we lived and my parents couldn't afford to feed eight kids, let alone pets. Grandpa and I would work the garden and then cook up the green beans or peas and eat them. We took a mound of dirt and together, helped it grow something good to eat. That was cool, not necessarily because we got something to eat, but I just liked hanging around my grandpa. I would sit with grandpa on the porch swing made of metal. I remember spray painting that swing for them with green paint when I was a teenager. Grandpa and I would watch people go by, sitting in that swing, and I was happy just listening to his stories. He would talk about being a cook in the army and a janitor at the VA hospital once he got out. Grandpa was a Scottish Mason. He was a 33rd level

Scottish Mason. I found that out when he died and it was a very big deal to be that level. The day he died I played in a chess tournament in high school and won a trophy. I went to see grandma and gave her the trophy in respect for grandpa.

I am sure I became a better person being around grandma and grandpa. My daughters have not had a grandpa figure to be around and that is why they like Al and Etta from our church so much. They are good substitutes for real grandparents. That is why it is so important for me to be a grandpa for my two girls, when they are parents. A safe environment helps to allow you to build trust in people. My daughters have never known an environment which was not trusting and safe. In that respect, I have done right by them and made their lives better than my experience was. Isn't that every parent's goal?

The time came for our first urologist appointment. Julie went with me. It was a typical office for a doctor. Magazines to read, a receptionist or office manager window where you make your co-payment on the insurance, and a certain smell that you think must be a certain type of cleaning solution that all these offices must use. We went back and I was told to give a urine sample. There was a mix-up and the records had not been ordered to be delivered, so after I gave the sample we had to wait about 30 minutes for that to happen.

In the examining room, I saw a picture of a stent. This was my first glimpse of what one looked like. It looked like a small tube with a pig tail. The pig tail would press on the bladder when you sit down. No wonder I had to go to the bathroom so often. Dr. Short came into the examining room and greeted us. He seemed nice and helpful. We asked if the kidney needed to come out. He was sure that surgery was

going to be needed, but wanted strong antibiotics in my system to reduce the size of any swollen tissues before doing the surgery. The procedure would be easier if the mass were smaller. Dr. Short did not know if the tumor was attached to the kidney or not. It was hard to see that from the scans. He took us both out to a backlit picture frame for the scans to be posted for viewing. He slipped one of the slides up into the gadget and you could see the kidney was huge still. You could also see the stent going down to the bladder.

“Can you tell if it is only cancer on the kidney and nowhere else?” I asked.

Dr. Short said, “Hmmm. From the pictures there are some fatty areas on the liver and it looks like you have some small gall stones, but we won’t know for sure until we operate.”

“Do you have an idea when that might be?”

“Well, we need to give it another month and have a scan done before your next visit. Then we can make plans after I see what the antibiotics do.”

Dr. Short asked if we had more questions, and we did not, so we went to the front and the next scan was scheduled for December 4th. A follow-up at the doctor’s office was scheduled for December 11th. This gave enough time for the radiologists to give their opinion and have the scans in Dr. Short’s office when we come. I had to go through Thanksgiving and into December not knowing what would happen. Back to limbo I went.

Usually we traveled during the holidays, so my kidney was a reason to stay home for one season. Julie started decorating the house and cooking and cleaning in preparation for the holidays. I remember trying not to get depressed for the holidays. I remember pretending I was a marathon runner, and only in the

eighth mile out of 26 total miles. It was hard. I would be having major surgery for the first time in my life, where I would be filleted like a fish and have my organs removed. This wasn't a good prospect or thought.

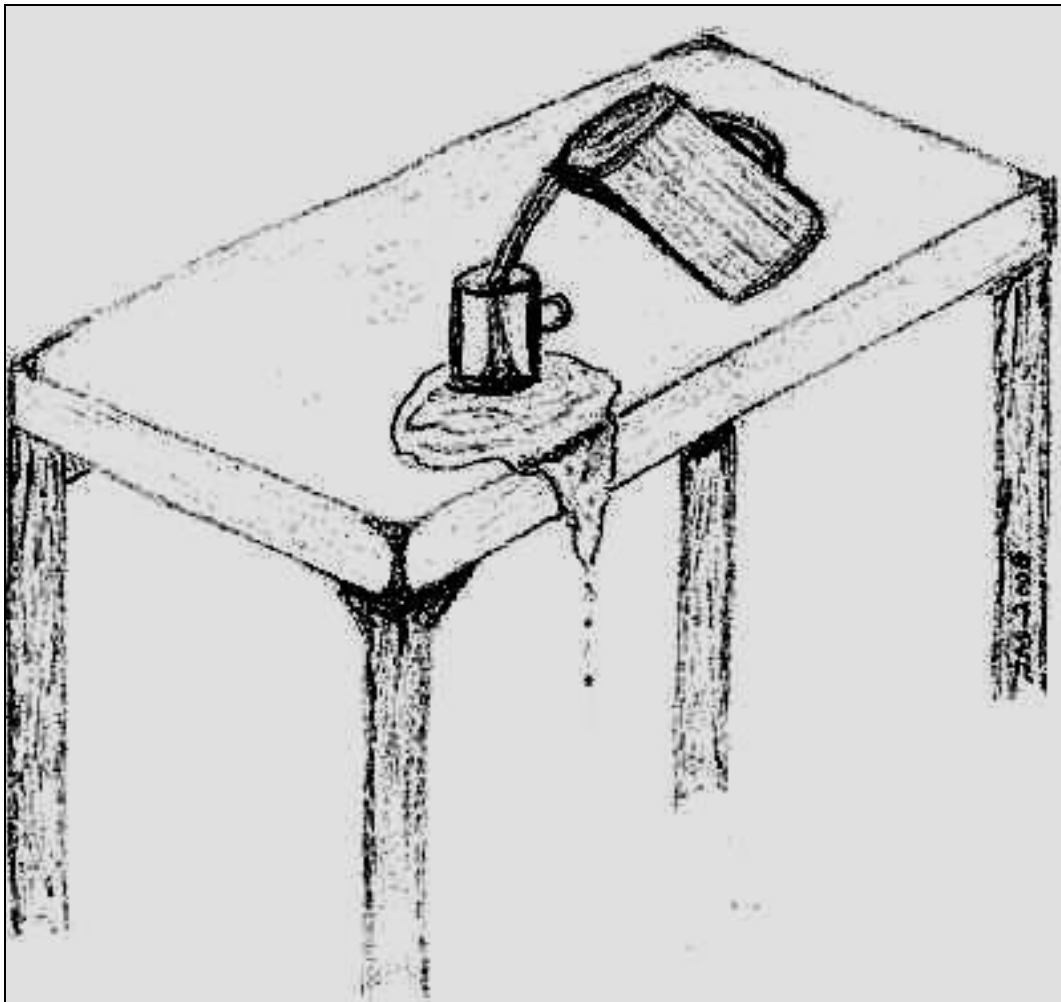
I planned to have the kidney surgery done in 2000, so the insurance could cover it as much as possible. Our deductible had been met and then some. I went over those target dates at the next appointment. There was a small chance that the tumor would go away and no surgery would be needed.

Thanksgiving came. We had a great Thanksgiving meal, with turkey and all the trimmings. Julie is a great cook. The women at work all say so too, when I bring in holiday treats she makes. This is one of the times we really use our formal dining room. Otherwise it is a hobby room or sewing room. The food smelled so good when it was cooking that day. I played Christmas

music on the stereo all day – not much on TV except parades in the morning and football in the afternoon.

Holidays with my wife and two children are traditional, with decorations and pets and fireplace scenes complete with me reading stories like “Lassie Come Home” or “‘Twas the Night Before Christmas”. It was not always so for me. When I was growing up, we did not have enough money for the decorations or pets, but we did have some presents from our relatives in the Danville area. I remember when I was around six or seven, our family got a wheel of cheddar cheese one winter. It was a big event. All of the families in the housing project got a cheese wheel that year. We had every imaginable dish with cheese. We had macaroni and cheddar cheese, cheeseburgers with cheddar cheese, hunks of cheddar cheese with our soup, pan-fried cheese sandwiches, and many more delicious cheddar cheese concoctions. Getting a wheel of cheese

is definitely non-traditional, but as I have gotten older, it is the people you have around and the people you come in contact with that matter, although it is a comfort to have a fireplace and piano music playing in the background by my daughter, like a Norman Rockwell painting. We made our home open that year to a needy family at church, but I guess the offer was too strange, and nobody took us up on it. I would have loved to have some people to feed and little children to play with.



“Providing a Urine Sample”

DECEMBER

After Thanksgiving, it was time for another MRI. It was December 4th, and this time I had a treat: I only had to drink one barium solution bottle instead of two. This is because the upper-GI (gastro-intestinal) was not needed and the lower-GI was enough to show the kidneys. It is the little things in life that make it all better. One bottle of bad tasting orange metal drink was so much easier to deal with. I was also feeling pretty good and it did not bother me as much as when I had peritonitis. The procedure was old hat now. I knew that a scan could be done without getting into a hospital gown just by getting rid of my belt. I also removed my glasses and wedding ring to not interfere with the magnet on the MRI.

I ended up being an interesting case for the hospital. They asked if they could also do a free MRV and if I

would mind. I said OK and it allowed them to get used to new features on the machine. The MRV is another test which can be performed with the same equipment as the MRI, but it allows the veins to show up more clearly on the scan. Blood flow in and out of the tumor was shown more clearly. Was the tumor using the kidney's blood flow or had it developed its own? The doctor would only remove the tumor and not the kidney if the kidney still had its own blood supply.

The technicians were always very nice and helpful and also very professional. I was glad that Baptist East Hospital existed and I felt comfortable they would take care of me. The lab technician put me on the conveyor for the MRI.

She said, "Mr. Snider, you just relax and breathe when we tell you to."

I did. I wished my veins were more easily accessible to the needle for the IV tube. I felt like I was inadequate somehow. It was my job to be a good patient after all. After this MRI was finished, there wasn't anyone to give an opinion other than the scans look like they were good quality. Julie and I went home and waited for the next doctor's appointment to review the results.

On December 11th, the doctor's appointment came. Julie and I went right to the correct parking lot and went up the elevator to the correct floor and into the office. The records were already there this time to review from the radiologists. We had to wait on the last visit for 30 minutes while the records were gathered. I still had to give a urine sample. I must have had a lot to drink, so this time I had too much for the little cup. This was going to get messy in a hurry. Imagine filling an eight ounce glass with a thirty-two ounce pitcher. At

some point you need another glass right? This was the same situation. I was clumsy about it, but managed not to get urine all over the bathroom. Whew! I felt like the star of a television sitcom. Since then, I have learned that the lab just needs a little bit, not full up to the brim.

I just left the sample per the nurse's instructions, but did get a moment to ask what they tested the urine for. She said the creatinine level. They wanted to make sure the other kidney was functioning normally and how much the kidney with the tumor was functioning as well. On a web site www.labtestsonline.org, I found this definition.

Creatinine is a waste product produced in your muscles from the breakdown of a compound called creatine. Creatine is part of the cycle that produces energy needed to contract your muscles and it as well as creatinine are produced at a relatively constant rate. Almost all creatinine is excreted by the kidneys, so blood levels are a good measure of how well your kidneys

are working. The quantity produced depends on the size of the person and their muscle mass. For this reason, creatinine concentrations will be slightly higher in men than in women and children.

Blood testing and/or urine samples were used in 2000.

I think only a blood sample is required today. That would eliminate the possibility of a urine catastrophe in their bathroom. Dr. Short looked at the chart and had us come over to his private office this time. He spent some time with us going over the new scan. Not much had changed. Dr. Short and six radiologists had looked into this case (since it was interesting) and could not tell me if the tumor could be separated off by itself through surgery or if the whole kidney needed to be removed.

By the way, one of my adrenal glands was up for grabs at this point also, since it works in conjunction with the kidney. Dr. Short said we could wait to see if

the tumor reduced more, which was unlikely. He recommended that the surgery be performed to really deal with the issue. I was ready. The earliest he could do it was late December or early January. We told him to make it the last week of December for insurance reasons. He said that was common and he would schedule it. It ended up being scheduled for Dec 27th, 2 days after Christmas. Dr. Short also mentioned that I could have the stent out at any time or wait for the surgery. I chose to wait. I had gotten used to the bladder spasm feeling and could control it now. Common sense told me that the fewer medical procedures I had, the better.

Julie and I went home after eating at the Red Lobster near the hospital. This was our small way of enjoying something in life when dealing with the medical issues. The meal was fine and we went home. A little over three weeks and I would have major

surgery. Now the anticipation was harder to handle, but I went to work and to church and tried to be normal about everything.

Julie decorated the house in Christmas fashion. We simplified the lights on the outside of the house since I would not feel like dealing with them when I came home from the hospital. We had a Christmas tree and put it in front of the window viewable by people on the street and in our home. I liked the angel we put on top of the tree. I liked the music this time of year. I liked the way the girls get excited this time of year. Our girls did not really understand what was going on, since they were only eleven and thirteen years old in December of 2000. They were preoccupied, working on Christmas gifts for relatives and themselves, rather than thinking about my surgery. It was like we lived dual lives – the medical story and the real life story.

During December we had a revival at my church. Buddy Barry was the evangelist. As part of doing everything I could, I went to the revival for that week as many nights as I could. There was singing and scripture and praising and prayer. Buddy is an energetic speaker. He is funny too. He brought a lady with him named Goldie. Goldie is a black woman and deeply religious. She would start the hymns and sing sometimes by herself. One chorus was about going to the river to be baptized. This reminded me of being baptized in Lake Decatur in Illinois. I was nineteen years old and wanted to get baptized as an adult, knowing full well what I was doing. Two weeks prior I was saved while driving in my car on Route 36 between Decatur and Danville one evening. Someone in the church made a loud noise, and my attention came back to the service. I had been the focus of many prayers during that revival by many people, and Goldie was one

of the people who prayed for me. She put her right hand on my stomach area and prayed for a healing. This made an impression on me from the fact that she did not know me but still wanted something good to happen in my future. She was bold and stepped out on faith.

I am not the greatest Christian example. I do not attend church enough. I do not pray enough. I do not read the Bible enough. It felt good during these last two months when I went to church, prayed and read the Bible. I just have a hard time keeping it up. I know I am not alone in that respect. I do not think God was punishing me, but I did not know at the time what he was doing either.

Christmas Eve was very special and beautiful. There was snow on the ground. Sometimes it is a hit or miss proposition to get snow in Oldham County, KY for Christmas. My wife and daughters were there, and I

had between Christmas and through the recovery off from work. This amounted to vacation time in December and short-term disability for the time off I would take in January 2001. On the mantle we had decorations and I built a fire in the fireplace. It was nice. Christmas came and was peaceful. There was the hubbub of noise around the opening of gifts and our cats playing with the wrapping paper. That was normal and expected. I was able to rest and gain some strength for the ordeal that lay ahead. I am not sure how people who are alone in the world cope with a major event like a surgery. My wife was heaven-sent during this time. She was the proverbial pillar to lean upon.

The day came to go to the hospital. I had to go early that morning and get prepared for surgery. This was only the second time for this process and each hospital is different in how they process people for surgery.

Once we were in the hospital prep area, it had many stations on the left and right sides, with curtains on every cubbyhole. There was a bed and equipment to monitor me and to provide a sedative to get ready for surgery. The nurse put in the IV and I remember telling the nurse that I was nervous.

She said, “Not for long,” since the medicine was going to kick into the IV soon and that would make me calm. The doctor and the anesthesiologist both came by and repeated a lot of the same questions the nurse posed. I guess that was for safety sake. I was kind of a prankster back then and still am, so I asked the doctor if I would be able to play the violin after the surgery.

He said, “You should be able to.”

I said, “That is good, I can’t today.”

Besides being a really old joke, I was getting groggy. My wife just shook her head. The doctor kept in good

humor about it though. It wasn't too long before I was being wheeled into surgery and left Julie to wait in the lounge to see how the surgery came out. Julie was told the surgery would take about 90 minutes. At the 90 minute mark, a person informed Julie that the mass was bigger than expected and it would take more time, but everything was going fine. Every 30 minutes after that, until over three hours went by, Julie was provided with updates. Luckily, my good friend Susie Isham came to the hospital shortly after I went into surgery. She stayed with Julie the entire three hours of the surgery. She even accompanied Julie to my soon-to-be recovery room. Our preacher was also with Julie, helping her wait. The surgery went well. As I started to wake up, I noticed two nurses tucking in blankets around my legs and Julie at the base of the bed.

My first words were "Isn't she pretty?"

The nurses, like robots, lifted their heads in unison, and turned to look at my wife, who was turning three shades of red. I was so glad to see her.

“He doesn’t have his glasses on” Julie said, trying to deflect the natural looks of expectation from the nurses.

It is not often I can embarrass Julie.

I took inventory again. Hands, toes, arms, legs, stomach, groin. Everything was there. Then I noticed something behind my back and was reaching around the left side of my body to see what it was. It was a bag for draining fluid from my abdomen. The tube went through my back and into my body around where the ribs stop and the stomach begins. The bag had a little fluid in it, but not much. I also felt the row of staples going around my whole left side. There were twenty-two of them, about one-half an inch apart. “I guess they had to remove it,” I thought. The bed was close to

the window. There was room for another bed in the room, but none was there. Private rooms are very nice. There must not have been a lot of patients, so I had a private room by default. There were some flowers and cards.

Julie asked, “How do you feel?”

“I feel groggy, but OK,” I said.

I was on the recovery wing for major surgeries: kidney surgery, hip joints, heart, lung, and stomach surgery patients recover on this floor. My room was one of many formed in a rectangle around the nurse’s station. I showed Julie my staples. She was properly amazed. I was supposed to just stay pretty quiet and recuperate for a few hours before attempting too much movement. No problem. I dosed off for most of it. The doctor came by to check up later that day and told us the kidney had to be removed. Along with it came

out the adrenal gland, the tumor, and surrounding tissue. As a bonus, the floating rib was removed on that side and sent off for testing to see if any cancer was in it. It is called a floating rib because it is not attached to the sternum. It is attached to another rib. The word cancer came up.

I said, “Did you get it all?”

Dr. Short said, “We think so. The tissue we took out will be analyzed and that way we will know for sure. That can take a week or two.”

I said, “What do you need me to do?”

Dr. Short responded, “Focus on going to the bathroom and resting and later tonight the nurses will see if you can walk.”

I said, “When does the bag come out?”

Dr. Short replied, “We will need to see if much drainage comes out and make a call on that tomorrow. Are there any more questions at this time?”

I said, “Yes. What is typical for a recovery time for what I have gone through?”

Dr. Short said, “Four to five days in the hospital and then three to six weeks at home. If there is nothing else, I need to see make my rounds.”

He left Julie and I alone and she needed to go home to the kids. I would not see Julie again until morning. Hospitals seem to be all about measuring vitals and pain management. I slept most of the time except when nurses needed me to take pills or take my blood pressure. I was able to get up out of bed to go to the bathroom, but it took all my energy to do that and my stomach area was really sore. About 3:00 in the morning, the nurses wanted me to walk down the hall.

They helped me up out of bed and told me this is what I needed to do to go home quicker. I was motivated, but not able. I went down the hall about thirty feet and back and I was in a lot of pain and discomfort. I forgot how many pain medications it took for the pain to go away, but it finally did. Pain medicine is addicting. I can see why. The slightest bump and I felt like I was being ripped apart. I would touch my stomach area and it was so swollen around the incision that it hurt to touch. The Frankenstein staples were cool though.

The next morning and I was still on a liquid diet. That sucked. Later I understood why. While the pain medication is in use, your bowels do not work. You build up in your intestines to the point that it feels worse than the pain of the surgery. You eventually go cold turkey on the pain medication to get some relief.

I was alone for hours each day with no visitors. I read everything. I watched a ton of TV in my room,

and then I heard it. There was screaming from an older woman down the hall. I found out she had hip surgery from a broken bone. That put me in my place as far as any complaining I might have wanted to do. Getting out of bed was the most exercise I managed. I could go to the restroom and, with Julie's help to keep me steady, I could take a shower. If I did not have her, a nurse would have done it. I am pretty shy, so it was a relief to have Julie around. I told Julie about the screaming woman and we talked about the girls and school. This was a painfully boring time for us both due to lack of activity.

When someone would come in to visit it broke up the day. Some people do not like visitors in the hospital. I am not one of those people. The preacher came and prayed. Brother Jim came back to be with us. Al and Etta came by to see how I was and pray with me for a speedy recovery. My good friend Susie at work

came by to see how I was doing. My boss came by and provided Julie with a Honeybaked Ham, all sliced and ready for sandwiches. That was wonderful for him to do. I got to show him my Frankenstein staples.

None of my family in Danville came to see me. None of them came. I guess I was the abnormal one that wanted visits instead of telling people to stay away. I guess I was not thinking about my hair or how I looked. I just wanted to see people. My girls did not visit me. They could not take seeing me in pain or sick. Both of them have a vision of me that does not include weakness. I would have been fine showing them that side of me, but they were not comfortable visiting me in the hospital.

Since I was encouraged to walk, that became my pastime. Sometimes I walked with Julie, sometimes alone. I walked several times a day. It helped to not stay in bed and build up fluid in the lungs. Also,

shaking my big belly like a bowlful of jelly would help to toughen me up around the incision. I would walk a little further each time, counting laps around the nurse's desk and down the hall. I stayed clear of the screaming woman's room. I am not sure why.



“Cowboy Shot with an Arrow” by Ed Coutts

On day two the doctor came by and looked in on me. Julie was there, and his mission was to pull out the drain tube that came out my back from my stomach area, since it was not leaking. Here is how it went. The doctor must have told Julie beforehand that this would hurt but only for a few seconds. Picture a cowboy being shot by an arrow in a western movie. He is lying there, propped up on a rock, and a friend must now break it and pull it out. The cowboy gets a strap of leather, bites down and takes it like a man. Owwww! My experience was similar. I did not get anything to bite down on, but Dr. Short choked up on the tube and pulled it half-way out. He choked up again on the tube. This time it came all of the way out. My reaction was immediate. This was the most painful thing I had ever experienced in my life. My muscle must have connected itself to this tube in that amount of time, and it was ripping during this jerk on the tube. I could not

speak for a few seconds – I had no air. My mouth was open. I made no sound but Ugh! My tongue was at the roof of my mouth and my mouth was open, but no words, just Ugh. I am glad that I was almost silent, because my loud voice would have made the screaming woman sit up and take notice. Julie was looking around the doctor at my face. By the look on her face she must have known it was coming. Man that hurt. The silver lining was that the drain bag was gone.

Step one in recovery was completed. Step two would be to get the staples out. Step three is to eat solid food. Step four is to get off the pain medication so my bowels can function. Step five is to go home. The staples came out right before I left the hospital. Solid food was only given to me one day before leaving the hospital. That first solid meal is one of the most appreciated meals I can think of. I never got off the pain

medication in the hospital, so the bowels did not move at all while I was there.

On the day of getting me out of the hospital, Julie pulled the car around while an orderly rolled me in a wheelchair to the outside door. Getting into the minivan was a challenge. The incision felt like it would rip open. Any jostling around caused me pain. Hitting bumps in the road caused pain. I moved from one hip to the other to minimize the impact of the bumps causing pain. Basically the whole ride home was painful. I made it home and was happy to be there. Getting up the stairs from the garage to the house was a new feat. I made that ok. My arm strength is good, so that added stability to the situation. I got into the house, looked around and sat on the sofa. I curled up and went to sleep. Since it was mid-morning, the girls were in school.

When I woke up all I could think about was that ham my boss had given us.

“Do we have any of that ham left?” I asked Julie.

“Yes. It is in the refrigerator. Have all you want.”

I headed into the kitchen and was now walking pretty well, just slowly. I grabbed some ham and just started eating it right out of the refrigerator until I was satisfied. I had some milk with it. Life was good. Having my tummy full was a good feeling, but then I remembered about not being able to go to the bathroom.

“Julie, I am not going to use pain medicine any more. I feel bloated and will need to use the restroom soon, so I don’t want it to lock me up.”

She said, “If you can handle the pain, it would be better for you.”

The girls came in later after school, and they gave me a hug on the couch. My family was complete again.

For another day (now several days of solid food), I was now hurting to use the restroom. My muscles finally kicked in and I finally got some relief. It is the small things like this you remember when recovering from major surgery. It was pure bliss!

Our neighbors came over to make sure we were ok, and to see if we needed anything. Julie was off on holiday break from teaching school, so she could go to the store and take care of things, so we really did not need anything at that point. New Year's Day was approaching.

JANUARY 2001

I remember it snowing in early January. I am the one in the family that deals with the snow on the driveway when it accumulates. The same goes for mowing the yard, but that would not need to be done for many months. For one or two inches we can just drive through it with the vehicles. More snow than that requires a shovel. Julie was in the house, so it must have been too early to go back to work or it was the weekend. Bessie is our dog. She always alerts us when people pull in the driveway. That is her job. She takes it seriously and always has to this day. Bessie is a clumber spaniel, the heaviest of the spaniels. I think it is an excuse for her to be fat, but anyway, she was barking.

Two men hopped out of a vehicle at the end of the drive and were coming up to shovel the driveway. I certainly could not manage it, so this was a true

blessing from two of the men from our church, Don and Ron. I was humbled by their service. This is the first time I ever experienced what a church family support was like. They were out there working and Julie made some hot cocoa in nice big white cups and I slowly took it out to them in my pajamas. They looked up and saw me. They stopped to drink the cocoa and talk about how I was doing.

Ron said, “Looks like you’re getting around pretty well.”

I said, “Yes. I just have to take it slow. I want to thank you guys for shoveling the snow. I was wondering how we would manage to do it.”

Don said, “Glad to help. You just go back in and rest.”

I took their advice, since it was very cold outside in my sock feet. The guys left the cocoa mugs on the

garage doorstep when they finished. They just left once they were done.

I believe that brick and mortar do not make a church. It is two men shoveling a walk for part of their family – their extended church family. It is providing for each other in times of need. It is learning about Jesus and being with other people who have the same Christian values and prayer and education in their lives. There have been many disasters since the year 2000, and many Christian acts of kindness have been shown. The 9/11 disaster in New York and more recently Hurricane Katrina in the Gulf Coast region are two horrific examples of tragedies. Shoveling a walk seems small in comparison to how people helped out in those situations, but it meant a great deal to me. It was a genuine kindness and it was appreciated. I won't forget it.



“Hot Cocoa” © Liz VanSteenBurgh at

www.BigStockPhoto.com

My focus shifted to preparing to go back to work in January. The doctor wanted me to go back only when I felt I could drive safely. I would know when that was, because the bumps in the road would be manageable for my twelve-inch around incision. I started to pick out more normal clothes to wear instead of sweats or jeans. I was looking at where my belt buckle was and noticed it was not centered any more on my body. The belt buckle was centered, but my navel wasn't. My navel moved! It moved one-half of an inch or so to the left due to my muscles having been cut and sewn together. How weird! Five years later, it is back in the right spot again. The skin stretched to accommodate.

I started to talk to people at work. My boss wanted to know how I was and return dates, etc. I told him it would be the 3rd week in January. We decided it would be that week and if I needed to make them short days, I could. Now that I had some goals, it was my job to

meet that date, just like when I managed projects and got them in on time at work. I would go out and sit outside when possible. I would ride in the car every chance I got, so that I could get used to the feeling on the incision. It felt better each week.

I went to church to see everyone there on Sundays. I thanked everyone for their prayers and ended up writing a letter to the church that was read one Sunday as a witness. Our preacher and close friend read the letter on a Sunday we would be traveling to see relatives. He said there was not a dry eye in the bunch.

I went to the urologist one last time before going back to work and had picked up a very nice pen set as a gift for him. I know we pay huge amounts of money through our insurance for doctors, but he was partially responsible for saving my life. Jesus made up the other component. This was a personal gift from me to him.

We found out that the mass taken out was four and one-

half pounds of flesh and blood. Dr. Short described it as volleyball size, and delivering it similar to a baby. He laid it on my torso while cutting the “cords” which in this case were blood vessels. No remaining cancer was found. Something else was odd too. The results came back that the tumor had started to crystallize and to cave in on itself. Jesus was at work all that time in my body, helping me to survive. Lungs, gall bladder and liver were all ok. Those would have been likely places for the cancer to spread to. I have to praise God for the healing he provided and medical talent he gave the surgeon. I thanked the doctor and he appreciated the nice pen.

I had to come back for scans at six months, one year, and two years. Then I would be totally clear. All of those scans came up cancer-free. The documentation in my records had mention of spine anomalies, but no cancer.

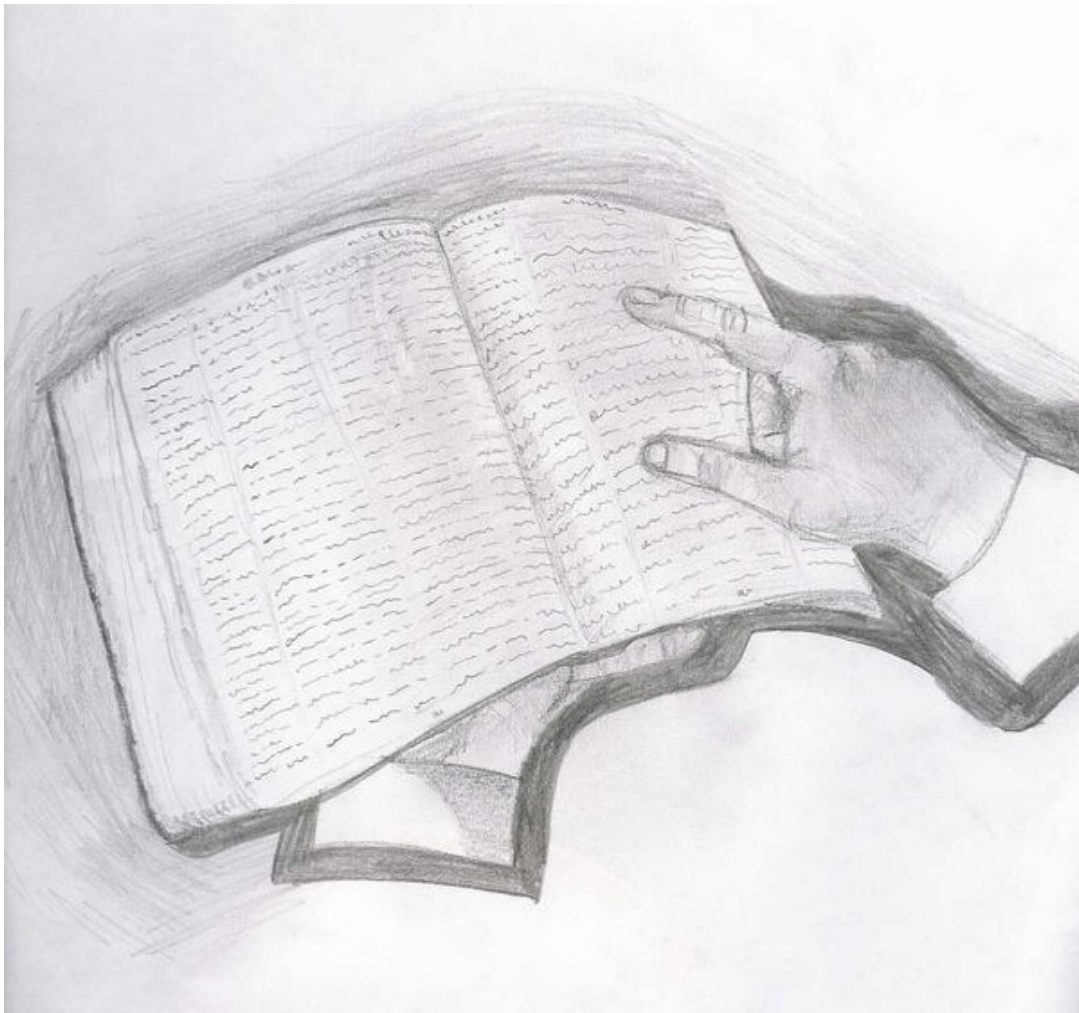
The day came to go back to work. I worked that day and not much new went on. People greeted me back. Then Eric came into my office and needed me to join him in one of the training rooms downstairs. They were working on some project plans and needed my help. I said, "Sure thing. I'll be right with you." We went down and as soon as I turned the corner, I started to see a crowd of people. Oh. Oh. Their must have been 80 people gathered around with me coming up the center towards a big cake. They were clapping and I was about in tears. They were all smiling at me. I remember that my friend Susie said something like "Welcome back!" and then they wanted me to speak. I saw the VP in charge of the IT team there smiling. I thanked God for letting me get healed and to be able to come back and then pointed to the lovely cake and thanked everyone for their support during this tough time. What a nice return! "Let's eat some cake!" Then

people starting milling around, talking to me, talking to each other, eating cake and some punch. I remember my boss and his boss welcoming me back and then Deb and Becca and Susie, who are three ladies in the data management group which we provide technical support for, welcoming me back. People wished me well and I had an incredible feeling of being appreciated.

I am thankful to work at a good company like YUM!Brands, Inc. We work hard and try to have fun too. The corporate culture is based on building people and their capabilities, and giving the customers what they want and deserve. It sounds much like the golden rule of treat people the way you want to be treated.

This concludes the tale of just one of my surgeries documented in this book. Even though the story is not over, I have remained cancer-free for 5 years and wish to give thanks.

Thanks goes to God for blessing me with health,
good friends, good neighbors, good people to work
with at YUM, a good church family including Eddy
Morris, Norman and Charlie Brown and Rocky Black at
Mt. Tabor United Methodist Church, two lovely
daughters and a great wife who helped me through it
all.



“Reading the Bible” by Emma Snider

TO MY KNEES AGAIN

As you get through one hurdle in life, more hurdles come, sooner or later. For the next 18 months, we worked on paying off the medical debts we accumulated. We owed thousands of dollars to doctors, hospitals and testing centers. We got behind on some payments, and that threw us into a new hurdle of debt management and collection agencies calling our house. Julie and I were at our wits end and did not want to declare bankruptcy, so we decided to use an agency to help us with debt management. The agency name we used I can't promote in this book, but I will give them a fictitious name called Organize-Our-Payments-Soon or OOPS.

Julie and I called OOPS and they were able to help us reduce our debt load by working with credit cards, and other lenders to write-off debt due to the cancer

situation and medical bills. OOPS did this for a fee of their own which was to be paid monthly for three years. We felt desperate, so we agreed and signed up. The bill collector's stopped calling, which was a major help to our sanity. We were able to get monthly payments worked out for the medical bills that piled up, and get on track. Any windfall we had during the next 18 months we paid on debt. We quickly got out of debt and paid OOPS also within two years.

One shortcoming of using a debt service is that any credit card debt that is written off is considered income and is therefore taxable. Our tax service did not keep up with this law, and we did not pay the right amount of taxes. Years later we ended up having to pay back-taxes with penalties once the IRS caught up with it. The tax service paid the penalties, but we still paid the taxes we owed. At least it was found during years that we could afford to pay them. We had about six months

to pay on the accumulated bills, and then a new hurdle cropped up.

In June of 2002, I woke up, slipped out the right side of the bed and starting walking towards the bathroom. I heard a “POP” from my back and went to my knees immediately. I tried to catch myself on the dresser, but it was too fast of a motion from standing to kneeling. I was on all fours in a split-second. It took a few seconds to figure out what had happened. In that few seconds, my mouth went dry, I could not focus my eyes and I felt like my body was broken into lower and upper halves.

I had some back problems between college years and specifically between the Junior and Senior years when I was hiking in Georgia’s mountains. I worked for a company that found gas leaks for power companies. I did survey work and sometimes walked transmission lines in the hills to see if any leaks were occurring. This was very strenuous work. In my late

teens and early twenties, I was 145 pounds, lean and mean. I used to be able to play tennis for hours on end, leg press over 200 pounds with each leg, and do over 45 pull ups. The job required me to ride in the back of pickups, so I would make a game out of getting in back of the truck by putting one hand on the side of the truck bed and leaping into the back, much like an old western movie when trick riders would get on horseback. I carried a twenty-pound steel rod for poking holes into the ground and then a meter for measuring the gas level in the ground. In one day, I would hike five miles up and down hills that sometimes required me to be a rock climber. One time when going up a hill, my lower back started complaining. Twenty years ago it was due to the physical labor, and more recently, I am convinced it is due to body weight.

The net effect of this recent back pain was that I could not walk correctly. This caused muscles to be

used in strange ways and all of my back muscles would hurt and stiffen up. Sitting was also uncomfortable. The sciatic nerve runs along from the lower back down through the back of each leg. Sciatica is usually caused by pressure on the sciatic nerve from a herniated disc (also referred to as a ruptured disc, pinched nerve, slipped disc, etc.). My symptoms were pain going through that nerve. At its most severe, it felt like my big toe was going to pop off. It also made my hamstring muscle tighten up. Back in my early twenties, I could stretch my legs and walk it off over time. Now, it would not be that easy.

I went to see my family doctor, since I learned through the kidney surgery that I needed to go sooner when something was going wrong with my body. She told me that I had a disc problem or a pinched nerve and referred me to a specialist. I will call the specialist Dr. Pain. Dr. Pain saw me six weeks after the pain

started. He was tremendously busy. Before the office visit I started to treat myself with stretching and posture exercises. It helped a little, but I knew something was physically wrong.

Dr. Pain's office was similar to many other medical offices. It was connected to a hospital where he did most of his surgical operations. One thing that was different was the faces of the patients. They were in pain, real pain. Back pain is definitely the worst pain I have ever suffered. It is equivalent to the pain I experienced when they pulled the tube from my back during the recuperation from kidney removal, only it does not go away. Once I got in to see Dr. Pain, the office visit was fine. He talked with me and scheduled an MRI at his local MRI testing facility nearby. About two weeks later, Dr. Pain reviewed the results with me. Now this was eight weeks into having this amount of pain. I could handle it better than most people, but it

was getting to me. Dr. Pain told me that he saw a congenital defect in my spine. Two of my discs were too close to the nerves since birth due to a narrowing of the spinal column. He believed I should have one of them worked on. The other one was not as severe, so he did not recommend any work on it. Dr. Pain prescribed pain killers and muscle-relaxers along with a steroid pack. The steroid pack was a set of pills that you take for seven days. Each day you took less and less of them. In two to three days, I felt a lot better, but when the seventh day hit, the pain returned. Surgery was two more weeks off. My feet were starting to be affected and I worried about nerve damage. It was like a constant firing of the sciatic nerve was happening and every five seconds I would feel it in my right foot. I would find a comfortable way to sit or stand or walk and it would last about five minutes when the feeling

came back. My patience was being tested to say the least. I asked God to help any way he saw fit.

I was reading the Bible at home to gain some spiritual guidance or relief. I found several passages that made me feel better. I sometimes worry that I am a fair-weather Christian. Only when times are tough do I sincerely seem to concentrate on being closer to God.

I always knew this was not life-threatening or as severe as the kidney surgery, so internally I just wanted the pain to go away without having to use pain killers. Sometimes getting hooked on those can be worse than the pain itself. Dilaudid is a derivative of morphine and was the only thing that would cut the pain long enough to be able to sleep all night. Taken orally, it is much more powerful than morphine and highly addictive. I can fully understand why people become addicted. I could not wait for the surgery to happen.

Julie and I went to the hospital on the day of the surgery. It reminded me of the kidney surgery. I went through similar tasks like the prep, getting the E.K.G. leads put on my body and having an IV set up. Once the anesthesiologist came and Dr. Pain showed up, it was time to get the show on the road. More knockout drops were put into the IV and all I remember is waking up in the recovery area. I remember it was a big open room and my room was not ready yet, so I was in a holding pattern. Nurses were checking on me from time to time. I was drifting in and out of sleep and just wanted water.

The incision was about four inches long on the lower spine area of my back. It was sore but nothing like the kidney surgery. No drainage bag with its tube to pull out and no staples were used.

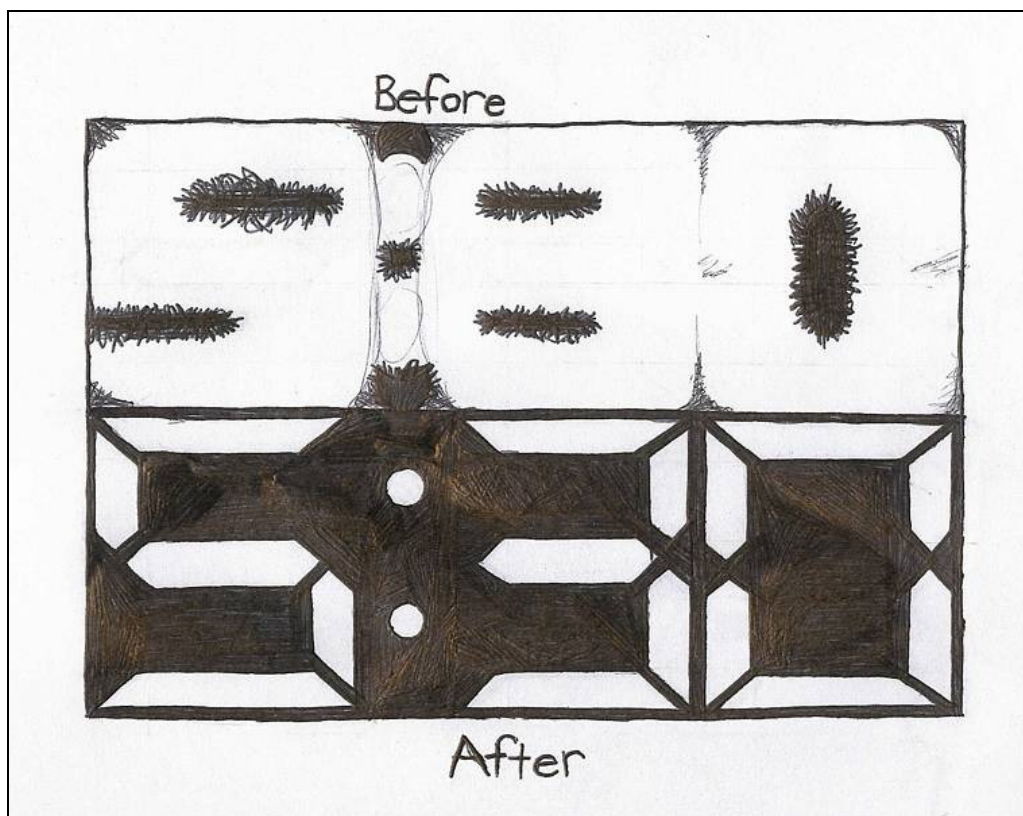
One piece of advice which I have not been able to follow myself: Get rid of your gut now! If you have a

pot gut, spare tire, love handles or whatever you want to call being shaped like a root beer barrel, do your self a favor - get rid of it now! This is what made recovery harder for me. Just doing the motion of a sit-up took a lot of strength from my arms. I found that sucking in my gut helped in almost all movements, making them easier. I had much less patience with being in the hospital and in three days, I was out of the hospital. I never did see Dr. Pain while recovering in the hospital. I had an appointment in two weeks to follow up on the surgery, so that would be my time to talk to him. I thought that was strange.

The same routine as the kidney surgery kicked in at home. My neighbors were much more involved with this surgery due to knowing more people who had this type of ailment or having chronic back pain themselves. They all wanted to see how I recovered and whether to potentially have surgery to get rid of their chronic pain.

I heard the stories of chiropractors pro and con, how when you have surgery you are never the same again, and the length of time it takes to truly recover being very long. This was during the summer, so the neighborhood pool parties would find me on the sidelines sitting in a chair and making sure I did not move too much.

After the un-ending hours of television, and two weeks going by, I went back to the doctor. I had no complaints and was going to go back to work. I could just as easily recuperate at my desk rather than being bored stiff at home. Dr. Pain agreed and let me go. No more appointments were necessary. I was feeling pretty good about this. Time would tell.



“Digital Clock” by Alyis Snider

IntraLASIK SURGERY

Having bad eyes since the fourth grade, Julie and I made the decision that while I was out recovering from back surgery, to go ahead and get IntraLASIK eye surgery done and only have one time out of work. I had wanted to do this for five years. Paying for it was always the problem. Feeling guilty about using the money instead of spending it on other things was another issue.

In Louisville, I found one of the leading doctors in town who had performed thousands of these types of operations. Dr. Maurice John was having a free seminar and I could actually see the operation being performed and talk to the person after the surgery. His was a class act from the word go. If all medical procedures were done this way, we would all be better off. It is elective surgery, so insurance does not pay for it. If it were cataract problems, it would be covered. I

had heard of radial keratotomy and horror stories about that, so I was skeptical but also hopeful at the same time. I learned quite a bit about flax seed which was a surprise to me. The eye doctor prescribed it before the surgery and encouraged use of it after the surgery. He explained that it acted as a lubricant and helped in recovery from eye surgery during the healing process.

There was a lecture portion of the two hour seminar, and then the live surgery was performed. There were two rooms and a viewing area between them where about ten of us could view at one time. Since there were around 20 people and the man was having both eyes done, we all got to see the operation being performed. There were video monitors with close-ups of the eye on them. The doctor and nurses and patient were all sterilized and the operation took 15 minutes per eye, 30 minutes total. The process was to cut a flap on the eye, pull it back, run the laser to remove a portion of

the cornea to correct your vision, and then put the flap back to heal. In the seminar, Dr. John told us about how the majority of complications (which were few to begin with) from this procedure are around cutting the flap. In most Lasik surgeries this is done with a blade at an angle. With my surgery it would be done with a laser and straight down. The complications were around tiny particles that might get in or under the blade. With the laser, that was a non-issue. I asked about recovery time and the doctor said, "One day and you will be fine. The first night, just go to bed early and the next day you will be normal." Eye drops were also given to a patient so that their eyes do not dry out.

I was impressed when the man who had the surgery on both eyes came out of the procedure, took his glasses off and threw them into a recycling glass jug never to use them again. He said he could see everyone in the room and was very happy! My only issue was

whether or not my eyes would be too bad for the procedure. I am near-sighted and have been since I was eleven, when my eyesight took a turn for the worse. My lenses on my glasses were very thick and I always looked for lighter-weight lenses so my glasses would not create dimples in my nose. I looked into contacts when the hard type was invented, but it did not work out. I also tried the soft type of contacts, but I do not blink very often and my eyes would dry out too much to wear them.

The examination was free as a part of the service, so I decided to proceed with that. Julie and I went to the eye examination together. We were still a bit nervous about spending the money. The examination was extremely thorough. It was like having three times the effort put into measuring my eyes than I had ever experienced before. Three people performed tests, and repeated some of them for accuracy. All these

measurements would be fed into a computer with the laser and that is how it would know the depths to cut. If my eyes were too bad, the procedure would not work on me. Other procedures might work, but this one wouldn't. I was still hopeful. After all the measurements in the two hour examination, the results were in and the eye doctor came in to give the verdict. Dr. John said I was a candidate, and my eyesight was poor, but that it could be corrected. The doctor gave no guarantees, but 20/20 eyesight was common. For someone in their forties, having reader-glasses for fine print was probably going to be necessary. Julie and I decided to go for it, and we scheduled the surgery for two weeks later, right before I was to come back to work from back surgery.

The comment that stuck in my mind was that I had three distinctly different people tell me off-hand that I had thick corneas. When I saw my wife later, I told her

she should be impressed because I had thick corneas, giving her a wink and a nod. She had a good comeback and said she might not be satisfied with a man who had thinner corneas after the procedure and we laughed.

I went to the IntraLASIK operation with some apprehension but more excitement than anything. I imagined not having glasses. That would be wonderful! The nurse put eye drops into my eyes to numb them. When I went into the operating room, I had to lie back and the doctor talked me through the process each step of the way. First they made the incision with the laser for the flap, then I felt a little pressure when the ring to hold my flap back was put it. They then took me to the other room for the other laser to cut on the cornea and the laser shaped it to make the eyesight correct. Since my back was very sore still, it was a little painful to get into the chair and recline, but that was the only pain I felt. It had nothing to do with

the surgery. The nicest thing was that one of the nurses was there to hold my hand during the operation. What service! No needles, no pain, no problems with the IntraLASIK surgery. The surgery only took 30 minutes total for both eyes! Zip Zap and it was done. When I met up with Julie, who had been watching as the operation was being performed, she asked if I could see. I looked around and it was great! I thanked the Lord for giving me clear sight through this process.

I put my glasses into the same bowl that the seminar patient puts his in. The glasses get recycled and are made available to people who need lenses in the U.S. and Africa. This marked a right of passage for me to the sighted world again. After thanking Dr. John and leaving the office, I had a one week follow-up examination and that was it. We went home and I went to bed not too long after that. I needed to have my eyes closed in a dark room overnight to heal properly.

I got up during the night to go to the bathroom. I could see everything in the room. This was amazing! I had never experienced this room without my glasses before. Could I see the digital clock? Yes! It was clear as a bell. This is great, I thought to myself at the time. Wow! I could actually see farther and with more clarity than with my glasses. I read the Charmin wrapper with my normal reading distance. Whoah!, too close. I had to put it almost at arm's length to read it clearly. Maybe

reading glasses were going to be needed like the doctor said. I went back to bed and went to sleep with a good feeling.

The next day I picked up some reading glasses from a local pharmacy for \$12. I tried on five pairs before the right magnification rating allowed me to see clearly up close. This was Saturday and I would be going back to work Monday. The next big test was working at a computer monitor most of the day. I had to adjust the distance a few times and clean the monitor, but it worked out fine. After about four hours, my eyes got tired. I tried not to push it and mixed up the days with monitor work and meetings.

The visit the following week was handled in a very professional manner. Dr. John re-tested my eyes and said the one eye was 20/20 and the other eye was 20/15 which is even better than 20/20. I was very pleased to be able to see an eye chart without glasses. Eventually,

I will take a new driver's reading test and have a picture taken without glasses. I knew many things like that would be different with my new eyes and I looked forward to it.

BACK TO WORK

Because I took two weeks to recover from the back surgery and the eye surgery only took one day, I did not take any short-term disability. That is a plus. There wasn't a cake this time when I returned. It was just work. Some people knew I was out and had some surgery, but not many. I kept it more close to the vest than the kidney surgery. I called to check on projects even during those two weeks of being out, so I wasn't backlogged with work when I returned.

People were amazed at recovering enough to come back in two weeks time. Many people talked about one month or even six month recovery times. A man I worked with had multiple back surgeries all the way up and down his spine. He was miserable most of the time and still required more surgery to correct his neck. I felt a little guilty for feeling somewhat OK.

Sitting was the main issue I fought. Finding a comfortable chair was another. I tried three chairs (kind of like Goldilocks and the three bears) until one was j-u-u-s-s-t right. Another man I worked with had similar surgery and he was out for eight weeks. He was told not to sit down for two weeks after coming back to work. I guess there is no standard treatment and recovery for back surgery. Not having the shooting pain down my right leg was a God-send. I walked a lot, since I tended to stiffen up. Things were going fine and as expected.

About three weeks later, my back took an abrupt turn for the worse. The pain started to come back. It was localized to my lower lumbar region and at least was not shooting down my leg. I thought to myself, “Oh, no. Not again.” I called the neurosurgeon’s office. Dr. Pain could not talk to me. The office nurse talked with him and relayed a message to take my pain

medicine and work through it. I gave that a try for the next week. It did not help. I called my family doctor and asked for a steroid pack. I went in to see the family doctor. The doctor's office prescribed it for me, but if I needed anything else they wanted me to go back to the neurosurgeon to get it, since I was technically still his patient recovering from back surgery. As before, the first two or three days I felt immediately better and by the seventh day, I felt crummy again.

A few days after that it got very bad. One morning, I could not move. I told Julie to call in sick for me. She called in. I literally was in so much pain that the only way I could move was an inch at a time, not knowing what falling off the bed would do to me. If I moved more than a little, I felt a shooting pain from my lower back up to my ribs and lungs.

I thought, "This is just great." Needing to go to the restroom, I had to try it.

Bracing myself for a massive amount of pain, I prayed, “Jesus help me.”

As I inched over to the right side of the bed and moved my right leg over, letting it drape over the edge of the bed, I was able to get my right arm under my mid-section to get ready to make the turn. In slow-motion, I turned my abdomen over to the right. I fell to the floor and hit my knees and hands and was positioned on all fours. It took the wind out of me. I literally crawled to the bathroom. It actually felt better in this position than in bed. Once I was able to move around a little, I could walk.

Julie got home in the evening and asked if I was any better. I was back in bed by that time, but laying on my side, just in case I needed to get out of bed for anything. I did not eat that day, did not sleep and did not watch television. I stared at the ceiling and petted any cats that came up to see what I was doing. By

night-time, I could not go to sleep due to the pain, and instead of taking pain medicine for it, Julie and I went to the emergency room.

The emergency room was full of people. There were people in their pajamas like me, with horrible looks on their faces, and some with blood on them. There was a gunshot victim, college kids, older people being led by their children through the medical process and then there was me with Julie. I was in a wheelchair and could sit in that pretty well. Hours went by and during that time I prayed a lot for me and Julie and for everyone else in the room.

The emergency room nurse called us back. The emergency room physician was quick and assessed the situation. He measured my movement and asked about my pain level. He said the area around my disc was inflamed and I needed to see the neurosurgeon. I told him that he would not see me without a referral

from a doctor. My family doctor would not do it, not wanting to treat me for what the neurosurgeon had done. I was in a catch-22 situation. The emergency room doctor gave me the referral. I was able to relax quite a bit once we got the referral taken care of.

It would be a week before I could see Dr. Pain. It must have been an embarrassment to him, having the emergency room call his office, scheduling me in. Evidently for this neurosurgeon, this was not the first time, based on the emergency room nurse's comments. The emergency room bore the burden of improper follow-up by the neurosurgeon, and I was paying the price in pain. Dr. Pain scheduled me for an MRI nearby his office. The appointment with Dr. Pain was on a Thursday. The MRI appointment was Tuesday.

Julie and I went to a local pharmacy and purchased a set of canes to use in the mean-time, since without them; I could not stand for any length of time. They

were the type that folded up like a blind person's cane. I could fit them easily in my car and used them at home.

Julie and I went to the MRI appointment, and I could barely stand up or lie down without shooting pain through my back. The only comfortable position in the car was to lean on one side in the passenger's seat. When we got there, we found out the machine was not working. There was a repair-man working on it, so we could stay to see if it got fixed, or go home. We stayed around for about an hour, and found out that the machine would be down for at least two more days and probably for a week. I called Dr. Pain's office to get scheduled at a different MRI testing facility. Dr. Pain was upset that we could not use his nearby facility and wanted to postpone my appointment until I could be tested there. This did not work for me. I needed treatment now! I did not agree with him, and needed

him to schedule the MRI at another facility. I wished that the doctor could understand my needs better. He relented, and set up the appointment at another testing site in another part of town for Wednesday evening.

Wednesday evening Julie and I went to the MRI center for the test. I looked like Arty Johnson from the old Laugh-In comedy show in how slowly I walked into the center. The ladies who ran the place took good care of me. That helped immensely. This time the MRI was not comfortable to be in, due to my back having spasms. I was expected to be still and breathe normally and hold my breath at appropriate times. I got through the test. Then it came time to get up off of the conveyor. I literally could not get up by myself from the table. The two technicians asked if they could help me up, but they did not want to hurt me.

“This will hurt anyway we do it, so go ahead and pull,” I said.

Each woman took an arm and pulled me off of the table. It hurt like hell. I landed on the ball of my right foot and steadied on it until my left foot got on the ground also. They had concerned looks on their faces, so I wanted to reassure them.

“I am fine. Thank you so much,” I said.

After about 10 minutes of slowly walking to the van, I got in and we started home. I was in so much pain that I could not stand the car ride. Julie pulled onto a side street and fed me the maximum dose of one of my pain meds. We sat and talked for almost thirty minutes until they had kicked in. Julie tried to miss bumps and take turns slowly, as we finally headed home.

On Thursday, I sat in the office for a very long time. Other people had appointments before me and mine was slated for 10:30 in the morning. The lady who sat next to me in the office said she was returning due to

getting a staff infection after surgery at the hospital. It almost killed her. I was sympathetic, since I was having complications also. People were coming in after me and going in before me. The doctor was running late. I got in at 12:30 in the afternoon. It made me think of a radio program I listened to many years prior where a lawyer billed his doctor for the waiting time wasted.

Once I finally got to meet with Dr. Pain, he saw me with a cane and did not have the nerve to mention my emergency room visit. He reviewed the MRI and told me the disc had become inflamed, which caused my pain. I was given two options. The first was to use muscle-relaxers and pain medicine and stay home until I felt better. Another was to go through physical therapy. I chose physical therapy. Dr. Pain wanted to go to lunch, and probably did not care for me anyway, so I got the bum's rush out of the office.

I forget how Julie and I found out, but later we were informed that the nearby testing facility was partially owned by Dr. Pain. We were very mad at him to begin with, but that really upset us, that my medical treatment would have been postponed possibly due to financial gain. The information came from the office personnel, so I believed it. It seemed to me a breach of ethics to make a profit off of my misery, especially being willing to postpone my treatment to line his pockets. These were horrible thoughts, I know, and I prayed about it several times, to forgive both of us.

At a hospital near where I live, they had a therapy section. This is where my therapy would be. I was able to drive, oddly enough, and sitting at work did not bother me too much. I went to the therapy evaluation and the workers there had me do some typical movements and bending this way and that way, pushing against them to measure muscle-strength, and

to test flexibility. Laying down and moving side-to-side was very painful. My muscle-strength was fine.

My therapy lasted for five weeks. Boy, am I glad I chose therapy. It was more expensive than pills, but worth every penny. Each session would last one hour. I had three sessions per week.

During the first week, I was given deep-tissue massage and some minor exercises to do. The massage felt wonderful. The exercises were meant to stretch my calf and hamstring muscles. The one exercise I still do to this day was to put my feet together and do push ups against a wall. If you do not move your feet, this stretches your legs and makes your back muscles in the upper torso get a work-out. During the first week, I also had heat-treatments to relax my muscles. My muscles had gotten so tight in my back from reacting to the jerks in my body that they needed to relax and stretch out.

People were so friendly and professional there. I would lie on my back and try to cross one leg over the other and twist side-to-side. It was not until the third week that I succeeded in doing that.

In the second week, more exercises were introduced during the hour-long sessions. I would use a long towel, grabbing each end with my hands and the other end around my foot and then lift up my leg. This was to stretch the calf and hamstring in the legs. The nurses would act as coaches, giving me encouragement to keep going and pushing it to the edge of pain, but not going any further.

It was around the fourth week when I noticed a big change. I could now start to work the reclining bicycle for 20 minutes and do rubber band exercises to move my legs in ways it would cause pain. I could feel my lower back muscles close to my spine kick-in and those

muscles were being built up to handle the normal stresses of daily movement.

During the time I was in physical therapy, I went to work as usual. On the fourth week of therapy, I used the canes less and less. First I went down to one cane. Then I was able to move around without them entirely.

At the end of the fifth week, on graduation day from therapy, the nurses re-tested my ability to move and it had improved tremendously. I was able to walk normally again.

Praise God!



“Journey” by Andy Lee

CONCLUSION

Over a two-year period, my journey took me from never having been in the hospital to being a veteran of medical procedures. My firsts over the two year period included an IV, taking morphine (although good BBQ is a close substitute for me), hospitalization, recovery from major surgery, living with CANCER, a church laying hands on me, people I do not know praying for me, MRI's and MRV's and CAT-scans, spine surgery, IntraLASIK eye surgery and physical therapy.

From the kidney surgery, my cancer was cleared up, but just as importantly, I renewed my faith. The pain took me to my knees, but I gained a new family – a church family. Sometimes God has a plan for you that you do not know about. For me, I had to be broken down to be able to accept help from others. It reminds me of movies about boot camp, when the drill

instructor keeps yelling and you keep exercising until you are exhausted. At that bottom point, you are much more receptive to change.

From the back surgery, the initial pain took me to my knees once again, but prayer and diligence and good physical therapy allowed me to walk again relatively quickly.

From the eye surgery, I ended up with perfect vision. Donating my glasses after having to wear them for 30 years was a liberating feeling.

A few key things I learned are worth repeating. If you have a spouse or significant other, do not take them for granted. Julie has been, and remains a crucial part of my life, and has been a blessing through my life. I remember having a big smile when she walked down the aisle to meet me when we got married, and I

remember how glad I was to see her after my kidney surgery and saying, “Isn’t she pretty!”

Another lesson was to be able to take help from others. Have the humility to let God work through you and others to do his will. Enjoy a church family and accept their help. Be a good member of a church family. You will not regret it.

A third lesson is that you have to manage your medical care these days. Medical professionals are busy, make mistakes, and you must be aware of your options.

A fourth lesson is to get off of the pain medicine as quickly as you can. Getting addicted or not being able to move your bowels is not a good set of options. A final major lesson is to use the best doctor you can find for eye surgery, or any surgery for that matter. My

eyesight is precious to me and was worth getting the best doctor to help me.

A final lesson was that God's timetable is not the same as mine. I wanted answers and results quicker than I could really absorb them. It has taken me five years to appreciate some of the lessons.

In the five years that have gone by, I have had clean cancer screenings. I still need to lose weight, but when my back started to act up again a couple of months ago, I went straight to dieting and physical therapy and more exercise versus surgery and have been able to ward off having a new experience with spine surgery. I guess you can teach old dogs new tricks.

I have also gone to church more, prayed more, and have become a better Christian. Most recently, I got back into teaching and just finished a night-class in statistics for MBA students at a Christian school

(Indiana Wesleyan University) nearby. Teaching college level has always been an interest of mine. I have felt more connected and appreciated than ever.

Again, it is time to give thanks. Praise God for all of these experiences and my ability to share them with you, the reader. Jesus helped this man see clearly, made this crippled man walk, and healed me of cancer which gave me my life back. From these experiences, I have these words for you. On my knees, I see clearly. With Jesus in your life, you can too!